
DIAGNOSTIC AND STATISTICAL
MANUAL OF
MENTAL DISORDERS

FIFTH EDITION
TEXT REVISION

DSM-5-TR™

AMERICAN PSYCHIATRIC ASSOCIATION

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Preface to DSM-5-TR

The American Psychiatric Association's *Diagnostic and Statistical Manual of Mental Disorders*, Fifth Edition, Text Revision (DSM-5-TR), is the first published revision to DSM-5. This revised manual integrates the original published DSM-5 diagnostic criteria with modifications (mostly for clarity) for over 70 disorders, comprehensively updated descriptive text accompanying each of the DSM disorders based on reviews of the literature since the publication of DSM-5, and the addition of a new diagnosis, prolonged grief disorder, and symptom codes for reporting suicidal and nonsuicidal self-injurious behavior. These changes differ from the scope of the prior text revision, DSM-IV-TR, in which the updates were confined almost exclusively to the text, leaving the diagnostic criteria virtually unchanged. This edition also integrates all prior online updates made to DSM-5 after its publication in 2013, in response to usage, specific scientific advances, and ICD-10-CM coding adjustments through an iterative revision process. Consequently, DSM-5-TR is the product of three separate revision processes, each one overseen by separate (but overlapping) groups of experts: the development of the original DSM-5 diagnostic criteria and text by the DSM-5 Task Force, published in 2013; updates to the DSM-5 diagnostic criteria and text by the DSM Steering Committee, which has overseen the iterative revision process; and fully updated text overseen by the Revision Subcommittee.

The clinical and research understanding of mental disorders continues to advance. As a result, most of the DSM-5-TR disorder texts have had at least some revision since the 9 years from original publication in DSM-5, with the overwhelming majority having had significant revisions. Sections of the text that were most extensively updated were Prevalence, Risk and Prognostic Factors, Culture-Related Diagnostic Issues, Sex- and Gender-Related Diagnostic Issues, Association With Suicidal Thoughts or Behavior, and Comorbidity. Also, for the first time ever, the entire DSM text has been reviewed and revised by a Work Group on Ethnoracial Equity and Inclusion to ensure appropriate attention to risk factors such as the experience of racism and discrimination, as well as to the use of non-stigmatizing language. For future periodic DSM-5-TR coding and other updates, see www.dsm5.org.

For benefit of reference in this manual, “DSM” refers generally to DSM as an entity, not specifying a particular edition (e.g., “Clinical training and experience are needed to use DSM for determining a clinical diagnosis.”). “DSM-5” refers to the entire set of currently approved criteria sets, disorders, other conditions, and content officially published in May 2013. “DSM-5-TR” refers to approved text in this current volume. Although the scope of the text revision did not include conceptual changes to the criteria sets or to other DSM-5 constructs, the need to make changes in certain diagnostic criteria sets for the purpose of clarification became apparent in conjunction with the text updates made across the book. Because the conceptual construct of criteria is unchanged, the criteria sets in DSM-5-TR that had their origins in DSM-5 are still referred to as “DSM-5-criteria.” The new diagnostic entity prolonged grief disorder is referred to

as a DSM-5-TR disorder, because of its addition in this volume.

The development of DSM-5-TR was a tremendous team effort. We are especially indebted to the tireless efforts of Wilson M. Compton, M.D., M.P.E., and Daniel S. Pine, M.D., as DSM-5 Text Revision Subcommittee Vice Chairs, as well as the more than 200 experts from across our field who did the lion's share of the work in the preparation of the text revision. We would also like to thank Paul Appelbaum, M.D., Chair of the DSM Steering

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Committee, along with the entire DSM Steering Committee, for their careful review of the text and criteria clarifications, and for making other helpful suggestions. Special gratitude goes to Ann M. Eng, DSM Managing Editor, for her timely shepherding of the DSM-5-TR development process from planning to completion and for her meticulous attention to detail, all critical to the success of this revision. We are grateful for the valuable contributions and help of Nitin Gogtay, M.D., Chief of the American Psychiatric Association Division of Research and Deputy Medical Director; Diana E. Clarke, Ph.D., Managing Director of Research and Senior Research Statistician/Epidemiologist; and Lamyaa H. Yousif, M.D., Ph.D., M.Sc., Senior DSM Operations Manager and Research Associate. We are thankful for the leadership of John McDuffie, Publisher, American Psychiatric Association Publishing, and the work of the following editorial and production staff at American Psychiatric Association Publishing in bringing this important work to fruition: Greg Kuny, Managing Editor, Books; Tammy Cordova, Graphic Design Manager; Andrew Wilson, Director of Production; Judy Castagna, Assistant Director of Production Services; Erika Parker, Acquisitions Editor; Alisa Riccardi, Senior Editor, Books; and Carrie Y. Farnham, Senior Editor, Books. Finally, we also recognize with appreciation Saul Levin, M.D., M.P.A., CEO and Medical Director of the American Psychiatric Association, for his advocacy and support of this comprehensive text revision.

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Preface to DSM-5

The American Psychiatric Association's *Diagnostic and Statistical Manual of Mental Disorders* (DSM) is a classification of mental disorders with associated criteria designed to facilitate more reliable diagnoses of these disorders. With successive editions over the past 60 years, it has become a standard reference for clinical practice in the mental health field. Since a complete description of the underlying pathological processes is not possible for most mental disorders, it is important to emphasize that the current diagnostic criteria are the best available description of how mental disorders are expressed and can be recognized by trained clinicians. DSM is intended to serve as a practical, functional, and flexible guide for organizing information that can aid in the accurate diagnosis and treatment of mental disorders. It is a tool for clinicians, an essential educational resource for students and practitioners, and a reference for researchers in the field.

Although this edition of DSM was designed first and foremost to be a useful guide to clinical practice, as an official nomenclature it must be applicable in a wide diversity of contexts. DSM has been used by clinicians and researchers from different orientations (biological, psychodynamic, cognitive, behavioral, interpersonal, family/systems), all of whom strive for a common language to communicate the essential characteristics of mental disorders presented by their patients. The information is of value to all professionals associated with various aspects of mental health care, including psychiatrists, other physicians, psychologists, social workers, nurses, counselors, forensic and legal specialists, occupational and rehabilitation therapists, and other health professionals. The criteria are concise and explicit and intended to facilitate an objective assessment of symptom presentations in a variety of clinical settings—inpatient, outpatient, partial hospital, consultation-liaison, clinical, private practice, and primary care—as well in general community epidemiological studies of mental disorders. DSM-5 is also a tool for collecting and communicating accurate public health statistics on mental disorder morbidity and mortality rates. Finally, the criteria and corresponding text serve as a textbook for students early in their profession who need a structured way to understand and diagnose mental disorders as well as for seasoned professionals encountering rare disorders for the first time. Fortunately, all of these uses are mutually compatible.

These diverse needs and interests were taken into consideration in planning DSM-5. The classification of disorders is harmonized with the World Health Organization's *International Classification of Diseases* (ICD), the official coding system used in the United States, so that the DSM criteria define disorders identified by ICD diagnostic names and code numbers. In DSM-5, both ICD-9-CM and ICD-10-CM codes (the latter scheduled for adoption in October 2015) are attached to the relevant disorders in the classification.

Although DSM-5 remains a categorical classification of separate disorders, we recognize that mental disorders do not always fit completely within the boundaries of a single disorder. Some symptom domains, such as depression and anxiety, involve multiple diagnostic categories and

may reflect common underlying vulnerabilities for a larger group of disorders. In recognition of this reality, the disorders included in DSM-5 were reordered into a revised organizational structure meant to stimulate new clinical perspectives. This new structure corresponds with the organizational arrangement of disorders planned for ICD-11 scheduled for release in 2015. Other enhancements have been introduced to promote ease of use across all settings:

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Representation of developmental issues related to diagnosis. The change in chapter organization better reflects a lifespan approach, with disorders more frequently diagnosed in childhood (e.g., neurodevelopmental disorders) at the beginning of the manual and disorders more applicable to older adulthood (e.g., neurocognitive disorders) at the end of the manual. Also, within the text, subheadings on development and course provide descriptions of how disorder presentations may change across the lifespan. Age-related factors specific to diagnosis (e.g., symptom presentation and prevalence differences in certain age groups) are also included in the text. For added emphasis, these age-related factors have been added to the criteria themselves where applicable (e.g., in the criteria sets for insomnia disorder and posttraumatic stress disorder, specific criteria describe how symptoms might be expressed in children). Likewise, gender and cultural issues have been integrated into the disorders where applicable.

- **Integration of scientific findings from the latest research in genetics and neuroimaging.** The revised chapter structure was informed by recent research in neuroscience and by emerging genetic linkages between diagnostic groups. Genetic and physiological risk factors, prognostic indicators, and some putative diagnostic markers are highlighted in the text. This new structure should improve clinicians' ability to identify diagnoses in a disorder spectrum based on common neurocircuitry, genetic vulnerability, and environmental exposures.
- **Consolidation of autistic disorder, Asperger's disorder, and pervasive developmental disorder into autism spectrum disorder.** Symptoms of these disorders represent a single continuum of mild to severe impairments in the two domains of social communication and restrictive repetitive behaviors/interests rather than being distinct disorders. This change is designed to improve the sensitivity and specificity of the criteria for the diagnosis of autism spectrum disorder and to identify more focused treatment targets for the specific impairments identified.
- **Streamlined classification of bipolar and depressive disorders.** Bipolar and depressive disorders are the most commonly diagnosed conditions in psychiatry. It was therefore important to streamline the presentation of these disorders to enhance both clinical and educational use. Rather than separating the definition of manic, hypomanic, and major depressive episodes from the definition of bipolar I disorder, bipolar II disorder, and major depressive disorder as in the previous edition, we included all of the component criteria within the respective criteria for each disorder. This approach will facilitate bedside diagnosis and treatment of these important disorders. Likewise, the explanatory notes for differentiating bereavement and major depressive disorder will provide far greater clinical guidance than was previously provided in the simple bereavement exclusion criterion. The new specifiers of anxious distress and mixed features are now fully described in the narrative on specifier variations that accompanies the criteria for these disorders.
- **Restructuring of substance use disorders for consistency and clarity.** The categories of substance abuse and substance dependence have been eliminated and replaced with an overarching new category of substance use disorders—with the specific substance used defining the specific disorders. "Dependence" has been easily confused with the term "addiction" when, in fact, the tolerance and withdrawal that previously defined dependence are actually very normal responses to prescribed medications that affect the central nervous system and do not necessarily indicate the presence of an addiction. By revising and clarifying these criteria in DSM-5, we hope to alleviate some of the widespread misunderstanding about these issues.

- **Enhanced specificity for major and mild neurocognitive disorders.** Given the explosion in neuroscience, neuropsychology, and brain imaging over the past 20 years, it was critical to convey the current state-of-the-art in the diagnosis of specific types of disorders that were previously referred to as the “dementias” or organic brain diseases. Biological markers identified by imaging for vascular and traumatic brain disorders and

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specific molecular genetic findings for rare variants of Alzheimer’s disease and Huntington’s disease have greatly advanced clinical diagnoses, and these disorders and others have now been separated into specific subtypes.

- **Transition in conceptualizing personality disorders.** Although the benefits of a more dimensional approach to personality disorders have been identified in previous editions, the transition from a categorical diagnostic system of individual disorders to one based on the relative distribution of personality traits has not been widely accepted. In DSM-5, the categorical personality disorders are virtually unchanged from the previous edition. However, an alternative “hybrid” model has been proposed in Section III to guide future research that separates interpersonal functioning assessments and the expression of pathological personality traits for six specific disorders. A more dimensional profile of personality trait expression is also proposed for a trait-specified approach.
- **Section III: new disorders and features.** A new section (Section III) has been added to highlight disorders that require further study but are not sufficiently well established to be a part of the official classification of mental disorders for routine clinical use. Dimensional measures of symptom severity in 13 symptom domains have also been incorporated to allow for the measurement of symptom levels of varying severity across all diagnostic groups. Likewise, the WHO Disability Assessment Schedule (WHODAS), a standard method for assessing global disability levels for mental disorders that is based on the International Classification of Functioning, Disability and Health (ICF) and is applicable in all of medicine, has been provided to replace the more limited Global Assessment of Functioning scale. It is our hope that as these measures are implemented over time, they will provide greater accuracy and flexibility in the clinical description of individual symptomatic presentations and associated disability during diagnostic assessments.
- **Online enhancements.** DSM-5 features online supplemental information. Additional cross-cutting and diagnostic severity measures are available online (www.psychiatry.org/dsm5), linked to the relevant disorders. In addition, the Cultural Formulation Interview, Cultural Formulation Interview—Informant Version, and supplementary modules to the core Cultural Formulation Interview are also included online at www.psychiatry.org/dsm5.

These innovations were designed by the leading authorities on mental disorders in the world and were implemented on the basis of their expert review, public commentary, and independent peer review. The 13 work groups, under the direction of the DSM-5 Task Force, in conjunction with other review bodies and, eventually, the APA Board of Trustees, collectively represent the global expertise of the specialty. This effort was supported by an extensive base of advisors and by the professional staff of the APA Division of Research; the names of everyone involved are too numerous to mention here but are listed in the Appendix. We owe tremendous thanks to those who devoted countless hours and invaluable expertise to this effort to improve the diagnosis of mental disorders.

We would especially like to acknowledge the chairs, text coordinators, and members of the 13 work groups, listed in the front of the manual, who spent many hours in this volunteer effort to improve the scientific basis of clinical practice over a sustained 6-year period. Susan K. Schultz, M.D., who served as text editor, worked tirelessly with Emily A. Kuhl, Ph.D., senior science writer and DSM-5 staff text editor, to coordinate the efforts of the work groups into a

cohesive whole. William E. Narrow, M.D., M.P.H., led the research group that developed the overall research strategy for DSM-5, including the field trials, that greatly enhanced the evidence base for this revision. In addition, we are grateful to those who contributed so much time to the independent review of the revision proposals, including Kenneth S. Kendler, M.D., and Robert Freedman, M.D., co-chairs of the Scientific Review Committee; John S. McIntyre, M.D., and Joel Yager, M.D., co-chairs of the Clinical and Public Health Committee; and Glenn Martin, M.D., chair of the APA Assembly

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DSM-5 Task Force Vice-Chair
December 19, 2012

DSM-5-TR Classification

Before each disorder name, ICD-10-CM codes are provided. Blank lines indicate that the ICD-10-CM code depends on the applicable subtype, specifier, or class of substance. For periodic DSM-5-TR coding and other updates, see www.dsm5.org.

Following chapter titles and disorder names, page numbers for the corresponding text or criteria are included in parentheses.

Note for all mental disorders due to another medical condition: Insert the name of the etiological medical condition within the name of the mental disorder due to [the medical condition]. The code and name for the etiological medical condition should be listed first immediately before the mental disorder due to the medical condition.

Neurodevelopmental Disorders (35)

Intellectual Developmental Disorders (37)

____	Intellectual Developmental Disorder (Intellectual Disability) (37)
	<i>Specify</i> current severity:
F70	Mild
F71	Moderate
F72	Severe
F73	Profound
F88	Global Developmental Delay (46)
F79	Unspecified Intellectual Developmental Disorder (Intellectual Disability) (46)

Communication Disorders (46)

F80.2	Language Disorder (47)
F80.0	Speech Sound Disorder (50)
F80.81	Childhood-Onset Fluency Disorder (Stuttering) (51)
	Note: Later-onset cases are diagnosed as F98.5 adult-onset fluency disorder.
F80.82	Social (Pragmatic) Communication Disorder (54)
F80.9	Unspecified Communication Disorder (56)

Autism Spectrum Disorder (56)

F84.0 Autism Spectrum Disorder (56)
Specify current severity: Requiring very substantial support, Requiring substantial support, Requiring support
Specify if: With or without accompanying intellectual impairment, With or without accompanying language impairment
Specify if: Associated with a known genetic or other medical condition or environmental factor
(**Coding note:** Use additional code to identify the associated genetic or other medical condition);
Associated with a neurodevelopmental, mental, or behavioral problem
Specify if: With catatonia (use additional code F06.1)

Attention-Deficit/Hyperactivity Disorder (68)

____.____ Attention-Deficit/Hyperactivity Disorder (68)
Specify if: In partial remission
Specify current severity: Mild, Moderate, Severe
Specify whether:

F90.2 Combined presentation

F90.0 Predominantly inattentive presentation

F90.1 Predominantly hyperactive/impulsive presentation

F90.8 Other Specified Attention-Deficit/Hyperactivity Disorder (76)

F90.9 Unspecified Attention-Deficit/Hyperactivity Disorder (76)

Specific Learning Disorder (76)

____.____ Specific Learning Disorder (76)
Specify current severity: Mild, Moderate, Severe
Specify if:

F81.0 With impairment in reading (specify if with word reading accuracy, reading rate or fluency, reading comprehension)

F81.81 With impairment in written expression (specify if with spelling accuracy, grammar and punctuation accuracy, clarity or organization of written expression)

F81.2 With impairment in mathematics (specify if with number sense, memorization of arithmetic facts, accurate or fluent calculation, accurate math reasoning)

Motor Disorders (85)

F82 Developmental Coordination Disorder (85)

F98.4 Stereotypic Movement Disorder (89)
Specify if: With self-injurious behavior, Without self-injurious behavior
Specify if: Associated with a known genetic or other medical condition, neurodevelopmental disorder, or environmental factor
Specify current severity: Mild, Moderate, Severe

Tic Disorders

F95.2 Tourette's Disorder (93)

F95.1 Persistent (Chronic) Motor or Vocal Tic Disorder (93)
Specify if: With motor tics only, With vocal tics only

F95.0	Provisional Tic Disorder (93)
F95.8	Other Specified Tic Disorder (98)
F95.9	Unspecified Tic Disorder (98)

Other Neurodevelopmental Disorders (99)

F88	Other Specified Neurodevelopmental Disorder (99)
F89	Unspecified Neurodevelopmental Disorder (99)

Schizophrenia Spectrum and Other Psychotic Disorders (101)

The following specifiers apply to Schizophrenia Spectrum and Other Psychotic Disorders where indicated:

^a*Specify* if: The following course specifiers are only to be used after a 1-year duration of the disorder: First episode, currently in acute episode; First episode, currently in partial remission; First episode, currently in full remission; Multiple episodes, currently in acute episode; Multiple episodes, currently in partial remission; Multiple episodes, currently in full remission; Continuous; Unspecified

^b*Specify* if: With catatonia (use additional code F06.1)

^c*Specify* current severity of delusions, hallucinations, disorganized speech, abnormal psychomotor behavior, negative symptoms, impaired cognition, depression, and mania symptoms

F21	Schizotypal (Personality) Disorder (104)
F22	Delusional Disorder ^{a,c} (104) <i>Specify</i> whether: Erotomantic type, Grandiose type, Jealous type, Persecutory type, Somatic type, Mixed type, Unspecified type <i>Specify</i> if: With bizarre content
F23	Brief Psychotic Disorder ^{b,c} (108) <i>Specify</i> if: With marked stressor(s), Without marked stressor(s), With peripartum onset
F20.81	Schizophreniform Disorder ^{b,c} (111) <i>Specify</i> if: With good prognostic features, Without good prognostic features
F20.9	Schizophrenia ^{a,b,c} (113)
____	Schizoaffective Disorder ^{a,b,c} (121) <i>Specify</i> whether:
F25.0	Bipolar type
F25.1	Depressive type
____	Substance/Medication-Induced Psychotic Disorder ^c (126) Note: For applicable ICD-10-CM codes, refer to the substance classes under Substance-Related and Addictive Disorders for the specific substance/medication-induced psychotic disorder. See also the criteria set and corresponding recording procedures in the manual for more information. Coding note: The ICD-10-CM code depends on whether or not there is a comorbid substance use disorder present for the same class of substance. In any case, an additional separate diagnosis of a substance use disorder is not given. <i>Specify</i> if: With onset during intoxication, With onset during withdrawal, With onset after medication use

____.	Psychotic Disorder Due to Another Medical Condition ^c (131) <i>Specify whether:</i>
F06.2	With delusions
F06.0	With hallucinations
F06.1	Catatonia Associated With Another Mental Disorder (Catatonia Specifier) (135)
F06.1	Catatonic Disorder Due to Another Medical Condition (136)
F06.1	Unspecified Catatonia (137) Note: Code first R29.818 other symptoms involving nervous and musculoskeletal systems.
F28	Other Specified Schizophrenia Spectrum and Other Psychotic Disorder (138)
F29	Unspecified Schizophrenia Spectrum and Other Psychotic Disorder (138)

Bipolar and Related Disorders (139)

The following specifiers apply to Bipolar and Related Disorders where indicated:

^a*Specify:* With anxious distress (*specify* current severity: mild, moderate, moderate-severe, severe); With mixed features; With rapid cycling; With melancholic features; With atypical features; With mood-congruent psychotic features; With mood-incongruent psychotic features; With catatonia (use additional code F06.1); With peripartum onset; With seasonal pattern

^b*Specify:* With anxious distress (*specify* current severity: mild, moderate, moderate-severe, severe); With mixed features; With rapid cycling; With peripartum onset; With seasonal pattern

____.	Bipolar I Disorder ^a (139)
____.	Current or most recent episode manic
F31.11	Mild
F31.12	Moderate
F31.13	Severe
F31.2	With psychotic features
F31.73	In partial remission
F31.74	In full remission
F31.9	Unspecified
F31.0	Current or most recent episode hypomanic
F31.71	In partial remission
F31.72	In full remission
F31.9	Unspecified
____.	Current or most recent episode depressed
F31.31	Mild
F31.32	Moderate
F31.4	Severe

F31.5	With psychotic features
F31.75	In partial remission
F31.76	In full remission
F31.9	Unspecified
F31.9	Current or most recent episode unspecified

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F31.81	Bipolar II Disorder (150) <i>Specify</i> current or most recent episode: Hypomanic ^b , Depressed ^a <i>Specify</i> course if full criteria for a mood episode are not currently met: In partial remission, In full remission <i>Specify</i> severity if full criteria for a major depressive episode are currently met: Mild, Moderate, Severe
F34.0	Cyclothymic Disorder (159) <i>Specify</i> if: With anxious distress (<i>specify</i> current severity: mild, moderate, moderate-severe, severe)
____.	Substance/Medication-Induced Bipolar and Related Disorder (162) Note: For applicable ICD-10-CM codes, refer to the substance classes under Substance-Related and Addictive Disorders for the specific substance/medication-induced bipolar and related disorder. See also the criteria set and corresponding recording procedures in the manual for more information. Coding note: The ICD-10-CM code depends on whether or not there is a comorbid substance use disorder present for the same class of substance. In any case, an additional separate diagnosis of a substance use disorder is not given. <i>Specify</i> if: With onset during intoxication, With onset during withdrawal, With onset after medication use
____.	Bipolar and Related Disorder Due to Another Medical Condition (166) <i>Specify</i> if:
F06.33	With manic features
F06.33	With manic- or hypomanic-like episode
F06.34	With mixed features
F31.89	Other Specified Bipolar and Related Disorder (168)
F31.9	Unspecified Bipolar and Related Disorder (169)
F39	Unspecified Mood Disorder (169)

Depressive Disorders ([177](#))

F34.81	Disruptive Mood Dysregulation Disorder (178)
____.	Major Depressive Disorder (183) <i>Specify:</i> With anxious distress (<i>specify</i> current severity: mild, moderate, moderate-severe, severe); With mixed features; With melancholic features; With atypical features; With mood-congruent psychotic features; With mood-incongruent psychotic features; With catatonia (use additional code F06.1); With peripartum onset; With seasonal pattern
____.	Single episode

F32.0	Mild
F32.1	Moderate
F32.2	Severe
F32.3	With psychotic features
F32.4	In partial remission
F32.5	In full remission
F32.9	Unspecified

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____.____ Recurrent episode

F33.0	Mild
F33.1	Moderate
F33.2	Severe
F33.3	With psychotic features
F33.41	In partial remission
F33.42	In full remission
F33.9	Unspecified
F34.1	Persistent Depressive Disorder (193)

Specify: With anxious distress (*specify* current severity: mild, moderate, moderate-severe, severe);
With atypical features

Specify if: In partial remission, In full remission

Specify if: Early onset, Late onset

Specify if: With pure dysthymic syndrome; With persistent major depressive episode; With intermittent major depressive episodes, with current episode; With intermittent major depressive episodes, without current episode

Specify current severity: Mild, Moderate, Severe

F32.81 Premenstrual Dysphoric Disorder ([197](#))

____.____ Substance/Medication-Induced Depressive Disorder ([201](#))

Note: For applicable ICD-10-CM codes, refer to the substance classes under Substance-Related and Addictive Disorders for the specific substance/medication-induced depressive disorder. See also the criteria set and corresponding recording procedures in the manual for more information.

Coding note: The ICD-10-CM code depends on whether or not there is a comorbid substance use disorder present for the same class of substance. In any case, an additional separate diagnosis of a substance use disorder is not given.

Specify if: With onset during intoxication, With onset during withdrawal, With onset after medication use

____.____ Depressive Disorder Due to Another Medical Condition ([206](#))

Specify if:

F06.31	With depressive features
F06.32	With major depressive–like episode
F06.34	With mixed features

F32.89	Other Specified Depressive Disorder (209)
F32.A	Unspecified Depressive Disorder (210)
F39	Unspecified Mood Disorder (210)

Anxiety Disorders (215)

F93.0	Separation Anxiety Disorder (217)
F94.0	Selective Mutism (222)
____.____	Specific Phobia (224)
	<i>Specify if:</i>
F40.218	Animal
F40.228	Natural environment
____.____	Blood-injection-injury

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F40.230	Fear of blood
F40.231	Fear of injections and transfusions
F40.232	Fear of other medical care
F40.233	Fear of injury
F40.248	Situational
F40.298	Other

F40.10	Social Anxiety Disorder (229)
	<i>Specify if:</i> Performance only

F41.0	Panic Disorder (235)
____.____	Panic Attack Specifier (242)

F40.00	Agoraphobia (246)
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F41.1	Generalized Anxiety Disorder (250)
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____.____	Substance/Medication-Induced Anxiety Disorder (255)
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Note: For applicable ICD-10-CM codes, refer to the substance classes under Substance-Related and Addictive Disorders for the specific substance/medication-induced anxiety disorder. See also the criteria set and corresponding recording procedures in the manual for more information.

Coding note: The ICD-10-CM code depends on whether or not there is a comorbid substance use disorder present for the same class of substance. In any case, an additional separate diagnosis of a substance use disorder is not given.

Specify if: With onset during intoxication, With onset during withdrawal, With onset after medication use

F06.4	Anxiety Disorder Due to Another Medical Condition (258)
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F41.8	Other Specified Anxiety Disorder (261)
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F41.9 Unspecified Anxiety Disorder (261)

Obsessive-Compulsive and Related Disorders (263)

The following specifier applies to Obsessive-Compulsive and Related Disorders where indicated:

^a*Specify if:* With good or fair insight, With poor insight, With absent insight/delusional beliefs

F42.2 Obsessive-Compulsive Disorder^a (265)

Specify if: Tic-related

F45.22 Body Dysmorphic Disorder^a (271)

Specify if: With muscle dysmorphia

F42.3 Hoarding Disorder^a (277)

Specify if: With excessive acquisition

F63.3 Trichotillomania (Hair-Pulling Disorder) (281)

F42.4 Excoriation (Skin-Picking) Disorder (284)

_____. Substance/Medication-Induced Obsessive-Compulsive and Related Disorder (287)

Note: For applicable ICD-10-CM codes, refer to the substance classes under Substance-Related and Addictive Disorders for the specific substance/medication-induced obsessive-compulsive and related disorder. See also the criteria set and corresponding recording procedures in the manual for more information.

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Coding note: The ICD-10-CM code depends on whether or not there is a comorbid substance use disorder present for the same class of substance. In any case, an additional separate diagnosis of a substance use disorder is not given.

Specify if: With onset during intoxication, With onset during withdrawal, With onset after medication use

F06.8 Obsessive-Compulsive and Related Disorder Due to Another Medical Condition (291)

Specify if: With obsessive-compulsive disorder–like symptoms, With appearance preoccupations, With hoarding symptoms, With hair-pulling symptoms, With skin-picking symptoms

F42.8 Other Specified Obsessive-Compulsive and Related Disorder (293)

F42.9 Unspecified Obsessive-Compulsive and Related Disorder (294)

Trauma- and Stressor-Related Disorders (295)

F94.1 Reactive Attachment Disorder (295)

Specify if: Persistent

Specify current severity: Severe

F94.2 Disinhibited Social Engagement Disorder (298)

Specify if: Persistent

Specify current severity: Severe

F43.10 Posttraumatic Stress Disorder (301)

Specify whether: With dissociative symptoms

Specify if: With delayed expression

____.____	Posttraumatic Stress Disorder in Individuals Older Than 6 Years (301)
____.____	Posttraumatic Stress Disorder in Children 6 Years and Younger (303)
F43.0	Acute Stress Disorder (313)
____.____	Adjustment Disorders (319)
	<i>Specify if:</i> Acute, Persistent (chronic)
	<i>Specify whether:</i>
F43.21	With depressed mood
F43.22	With anxiety
F43.23	With mixed anxiety and depressed mood
F43.24	With disturbance of conduct
F43.25	With mixed disturbance of emotions and conduct
F43.20	Unspecified
F43.8	Prolonged Grief Disorder (322)
F43.8	Other Specified Trauma- and Stressor-Related Disorder (327)
F43.9	Unspecified Trauma- and Stressor-Related Disorder (328)

Dissociative Disorders (329)

F44.81	Dissociative Identity Disorder (330)
F44.0	Dissociative Amnesia (337)
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	<i>Specify if:</i>
F44.1	With dissociative fugue
F48.1	Depersonalization/Derealization Disorder (343)
F44.89	Other Specified Dissociative Disorder (347)
F44.9	Unspecified Dissociative Disorder (348)

Somatic Symptom and Related Disorders (349)

F45.1	Somatic Symptom Disorder (351)
	<i>Specify if:</i> With predominant pain
	<i>Specify if:</i> Persistent
	<i>Specify current severity:</i> Mild, Moderate, Severe
F45.21	Illness Anxiety Disorder (357)
	<i>Specify whether:</i> Care-seeking type, Care-avoidant type

_____.	Functional Neurological Symptom Disorder (Conversion Disorder) (360) <i>Specify</i> if: Acute episode, Persistent <i>Specify</i> if: With psychological stressor (specify stressor), Without psychological stressor <i>Specify</i> symptom type:
F44.4	With weakness or paralysis
F44.4	With abnormal movement
F44.4	With swallowing symptoms
F44.4	With speech symptom
F44.5	With attacks or seizures
F44.6	With anesthesia or sensory loss
F44.6	With special sensory symptom
F44.7	With mixed symptoms
F54	Psychological Factors Affecting Other Medical Conditions (364) <i>Specify</i> current severity: Mild, Moderate, Severe, Extreme
_____.	Factitious Disorder (367) <i>Specify</i> : Single episode, Recurrent episodes
F68.10	Factitious Disorder Imposed on Self
F68.A	Factitious Disorder Imposed on Another
F45.8	Other Specified Somatic Symptom and Related Disorder (370)
F45.9	Unspecified Somatic Symptom and Related Disorder (370)

Feeding and Eating Disorders (371)

The following specifiers apply to Feeding and Eating Disorders where indicated:

^a*Specify* if: In remission

^b*Specify* if: In partial remission, In full remission

^c*Specify* current severity: Mild, Moderate, Severe, Extreme

_____.	Pica ^a (371)
F98.3	In children
F50.89	In adults

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F98.21	Rumination Disorder ^a (374)
F50.82	Avoidant/Restrictive Food Intake Disorder ^a (376)
_____.	Anorexia Nervosa ^{b,c} (381) <i>Specify</i> whether:
F50.01	Restricting type

F50.02	Binge-eating/purging type
F50.2	Bulimia Nervosa ^{b,c} (387)
F50.81	Binge-Eating Disorder ^{b,c} (392)
F50.89	Other Specified Feeding or Eating Disorder (396)
F50.9	Unspecified Feeding or Eating Disorder (397)

Elimination Disorders (399)

F98.0	Enuresis (399) <i>Specify</i> whether: Nocturnal only, Diurnal only, Nocturnal and diurnal
F98.1	Encopresis (402) <i>Specify</i> whether: With constipation and overflow incontinence, Without constipation and overflow incontinence
____.____	Other Specified Elimination Disorder (405)
N39.498	With urinary symptoms
R15.9	With fecal symptoms
____.____	Unspecified Elimination Disorder (405)
R32	With urinary symptoms
R15.9	With fecal symptoms

Sleep-Wake Disorders (407)

The following specifiers apply to Sleep-Wake Disorders where indicated:

^a*Specify* if: Episodic, Persistent, Recurrent

^b*Specify* if: Acute, Subacute, Persistent

^c*Specify* current severity: Mild, Moderate, Severe

F51.01	Insomnia Disorder ^a (409) <i>Specify</i> if: With mental disorder, With medical condition, With another sleep disorder
F51.11	Hypersomnolence Disorder ^{b,c} (417) <i>Specify</i> if: With mental disorder, With medical condition, With another sleep disorder
____.____	Narcolepsy ^c (422) <i>Specify</i> whether:
G47.411	Narcolepsy with cataplexy or hypocretin deficiency (type 1)
G47.419	Narcolepsy without cataplexy and either without hypocretin deficiency or hypocretin unmeasured (type 2)

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G47.421	Narcolepsy with cataplexy or hypocretin deficiency due to a medical
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condition
G47.429 Narcolepsy without cataplexy and without hypocretin deficiency due to a medical condition

Breathing-Related Sleep Disorders (429)

G47.33 Obstructive Sleep Apnea Hypopnea^c (429)

____.____ Central Sleep Apnea (435)

Specify current severity

Specify whether:

G47.31 Idiopathic central sleep apnea

R06.3 Cheyne-Stokes breathing

G47.37 Central sleep apnea comorbid with opioid use

Note: First code opioid use disorder, if present.

____.____ Sleep-Related Hypoventilation (439)

Specify current severity

Specify whether:

G47.34 Idiopathic hypoventilation

G47.35 Congenital central alveolar hypoventilation

G47.36 Comorbid sleep-related hypoventilation

____.____ Circadian Rhythm Sleep-Wake Disorders^a (443)

Specify whether:

G47.21 Delayed sleep phase type (444)

Specify if: Familial, Overlapping with non-24-hour sleep-wake type

G47.22 Advanced sleep phase type (446)

Specify if: Familial

G47.23 Irregular sleep-wake type (447)

G47.24 Non-24-hour sleep-wake type (448)

G47.26 Shift work type (450)

G47.20 Unspecified type

Parasomnias (451)

____.____ Non-Rapid Eye Movement Sleep Arousal Disorders (452)

Specify whether:

F51.3 Sleepwalking type

Specify if: With sleep-related eating, With sleep-related sexual behavior (sexsomnia)

F51.4 Sleep terror type

F51.5	Nightmare Disorder ^{b,c} (457) <i>Specify if:</i> During sleep onset <i>Specify if:</i> With mental disorder, With medical condition, With another sleep disorder
G47.52	Rapid Eye Movement Sleep Behavior Disorder (461)

G25.81	Restless Legs Syndrome (464)
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____.	Substance/Medication-Induced Sleep Disorder (468) Note: For applicable ICD-10-CM codes, refer to the substance classes under Substance-Related and Addictive Disorders for the specific substance/medication-induced sleep disorder. See also the criteria set and corresponding recording procedures in the manual for more information. Coding note: The ICD-10-CM code depends on whether or not there is a comorbid substance use disorder present for the same class of substance. In any case, an additional separate diagnosis of a substance use disorder is not given. <i>Specify whether:</i> Insomnia type, Daytime sleepiness type, Parasomnia type, Mixed type <i>Specify if:</i> With onset during intoxication, With onset during withdrawal, With onset after medication use
G47.09	Other Specified Insomnia Disorder (475)
G47.00	Unspecified Insomnia Disorder (475)
G47.19	Other Specified Hypersomnolence Disorder (475)
G47.10	Unspecified Hypersomnolence Disorder (476)
G47.8	Other Specified Sleep-Wake Disorder (476)
G47.9	Unspecified Sleep-Wake Disorder (476)

Sexual Dysfunctions (477)

The following specifiers apply to Sexual Dysfunctions where indicated:

^a*Specify whether:* Lifelong, Acquired

^b*Specify whether:* Generalized, Situational

^c*Specify current severity:* Mild, Moderate, Severe

F52.32	Delayed Ejaculation ^{a,b,c} (478)
F52.21	Erectile Disorder ^{a,b,c} (481)
F52.31	Female Orgasmic Disorder ^{a,b,c} (485) <i>Specify if:</i> Never experienced an orgasm under any situation
F52.22	Female Sexual Interest/Arousal Disorder ^{a,b,c} (489)
F52.6	Genito-Pelvic Pain/Penetration Disorder ^{a,c} (493)
F52.0	Male Hypoactive Sexual Desire Disorder ^{a,b,c} (498)
F52.4	Premature (Early) Ejaculation ^{a,b,c} (501)
____.	Substance/Medication-Induced Sexual Dysfunction ^c (504)

Note: For applicable ICD-10-CM codes, refer to the substance classes under Substance-Related and Addictive Disorders for the specific substance/medication-induced sexual dysfunction. See also the criteria set and corresponding recording procedures in the manual for more information.

Coding note: The ICD-10-CM code depends on whether or not there is a comorbid substance use disorder present for the same class of substance. In any case, an additional separate diagnosis of a substance use disorder is not given.

Specify if: With onset during intoxication, With onset during withdrawal, With onset after medication use

- F52.8** Other Specified Sexual Dysfunction (509)
- F52.9** Unspecified Sexual Dysfunction (509)

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Gender Dysphoria (511)

The following specifier and note apply to Gender Dysphoria where indicated:

^a*Specify* if: With a disorder/difference of sex development

^b**Note:** Code the disorder/difference of sex development if present, in addition to gender dysphoria.

- ____.____ Gender Dysphoria (512)
- F64.2** Gender Dysphoria in Children^{a,b}
- F64.0** Gender Dysphoria in Adolescents and Adults^{a,b}
- Specify* if: Posttransition
- F64.8** Other Specified Gender Dysphoria (520)
- F64.9** Unspecified Gender Dysphoria (520)

Disruptive, Impulse-Control, and Conduct Disorders (521)

- F91.3** Oppositional Defiant Disorder (522)
- Specify* current severity: Mild, Moderate, Severe
- F63.81** Intermittent Explosive Disorder (527)
- ____.____ Conduct Disorder (530)
- Specify* if: With limited prosocial emotions
- Specify* current severity: Mild, Moderate, Severe
- Specify* whether:
- F91.1** Childhood-onset type
- F91.2** Adolescent-onset type
- F91.9** Unspecified onset
- F60.2** Antisocial Personality Disorder (537)
- F63.1** Pyromania (537)
- F63.2** Kleptomania (539)

F91.8	Other Specified Disruptive, Impulse-Control, and Conduct Disorder (541)
F91.9	Unspecified Disruptive, Impulse-Control, and Conduct Disorder (541)

Substance-Related and Addictive Disorders (543)

Substance-Related Disorders (544)

Alcohol-Related Disorders (553)

____	Alcohol Use Disorder (553)
	<i>Specify if:</i> In a controlled environment
	<i>Specify current severity/remission:</i>
F10.10	Mild
F10.11	In early remission
F10.11	In sustained remission
	xl
F10.20	Moderate
F10.21	In early remission
F10.21	In sustained remission
F10.20	Severe
F10.21	In early remission
F10.21	In sustained remission
____	Alcohol Intoxication (561)
F10.120	With mild use disorder
F10.220	With moderate or severe use disorder
F10.920	Without use disorder
____	Alcohol Withdrawal (564)
	Without perceptual disturbances
F10.130	With mild use disorder
F10.230	With moderate or severe use disorder
F10.930	Without use disorder
	With perceptual disturbances
F10.132	With mild use disorder
F10.232	With moderate or severe use disorder
F10.932	Without use disorder

____ Alcohol-Induced Mental Disorders (567)
Note: Disorders are listed in their order of appearance in the manual.

^a*Specify* With onset during intoxication, With onset during withdrawal

^b*Specify* if: Acute, Persistent

^c*Specify* if: Hyperactive, Hypoactive, Mixed level of activity

____.	Alcohol-Induced Psychotic Disorder ^a (126)
F10.159	With mild use disorder
F10.259	With moderate or severe use disorder
F10.959	Without use disorder
____.	Alcohol-Induced Bipolar and Related Disorder ^a (162)
F10.14	With mild use disorder
F10.24	With moderate or severe use disorder
F10.94	Without use disorder
____.	Alcohol-Induced Depressive Disorder ^a (201)
F10.14	With mild use disorder
F10.24	With moderate or severe use disorder
F10.94	Without use disorder
____.	Alcohol-Induced Anxiety Disorder ^a (255)
F10.180	With mild use disorder
F10.280	With moderate or severe use disorder
F10.980	Without use disorder
____.	Alcohol-Induced Sleep Disorder ^a (468)
	<i>Specify</i> whether Insomnia type
F10.182	With mild use disorder
xli	
F10.282	With moderate or severe use disorder
F10.982	Without use disorder
____.	Alcohol-Induced Sexual Dysfunction ^a (504)
	<i>Specify</i> if: Mild, Moderate, Severe
F10.181	With mild use disorder
F10.281	With moderate or severe use disorder
F10.981	Without use disorder
____.	Alcohol Intoxication Delirium ^{b,c} (672)
F10.121	With mild use disorder
F10.221	With moderate or severe use disorder

F10.921	Without use disorder
____.	Alcohol Withdrawal Delirium ^{b,c} (673)
F10.131	With mild use disorder
F10.231	With moderate or severe use disorder
F10.931	Without use disorder
____.	Alcohol-Induced Major Neurocognitive Disorder (712)
	<i>Specify</i> if: Persistent
____.	Amnestic-confabulatory type
F10.26	With moderate or severe use disorder
F10.96	Without use disorder
____.	Nonamnestic-confabulatory type
F10.27	With moderate or severe use disorder
F10.97	Without use disorder
____.	Alcohol-Induced Mild Neurocognitive Disorder (712)
	<i>Specify</i> if: Persistent
F10.188	With mild use disorder
F10.288	With moderate or severe use disorder
F10.988	Without use disorder
F10.99	Unspecified Alcohol-Related Disorder (568)
	Caffeine-Related Disorders (569)
F15.920	Caffeine Intoxication (569)
F15.93	Caffeine Withdrawal (571)
____.	Caffeine-Induced Mental Disorders (574)
	Note: Disorders are listed in their order of appearance in the manual.
	<i>Specify</i> With onset during intoxication, With onset during withdrawal, With onset after medication use.
	Note: When taken over the counter, substances in this class can also induce the relevant substance-induced mental disorder.
F15.980	Caffeine-Induced Anxiety Disorder (255)
F15.982	Caffeine-Induced Sleep Disorder (468)
	<i>Specify</i> whether Insomnia type, Daytime sleepiness type, Mixed type
F15.99	Unspecified Caffeine-Related Disorder (574)

Cannabis-Related Disorders (575)

____.	Cannabis Use Disorder (575)
	<i>Specify</i> if: In a controlled environment

	Specify current severity/remission:
F12.10	Mild
F12.11	In early remission
F12.11	In sustained remission
F12.20	Moderate
F12.21	In early remission
F12.21	In sustained remission
F12.20	Severe
F12.21	In early remission
F12.21	In sustained remission
____.	Cannabis Intoxication (582)
	Without perceptual disturbances
F12.120	With mild use disorder
F12.220	With moderate or severe use disorder
F12.920	Without use disorder
	With perceptual disturbances
F12.122	With mild use disorder
F12.222	With moderate or severe use disorder
F12.922	Without use disorder
____.	Cannabis Withdrawal (584)
F12.13	With mild use disorder
F12.23	With moderate or severe use disorder
F12.93	Without use disorder
____.	Cannabis-Induced Mental Disorders (586)
	Note: Disorders are listed in their order of appearance in the manual.
	^a Specify With onset during intoxication, With onset during withdrawal, With onset after medication use. Note: When prescribed as medication, substances in this class can also induce the relevant substance-induced mental disorder.
	^b Specify if: Acute, Persistent
	^c Specify if: Hyperactive, Hypoactive, Mixed level of activity
____.	Cannabis-Induced Psychotic Disorder ^a (126)
F12.159	With mild use disorder
F12.259	With moderate or severe use disorder
F12.959	Without use disorder
____.	Cannabis-Induced Anxiety Disorder ^a (255)
F12.180	With mild use disorder

F12.280 With moderate or severe use disorder

F12.980 Without use disorder

____.____ Cannabis-Induced Sleep Disorder^a (468)

Specify whether Insomnia type, Daytime sleepiness type, Mixed type

F12.188 With mild use disorder

F12.288 With moderate or severe use disorder

xliii

F12.988 Without use disorder

____.____ Cannabis Intoxication Delirium^{b,c} (672)

F12.121 With mild use disorder

F12.221 With moderate or severe use disorder

F12.921 Without use disorder

F12.921 Pharmaceutical Cannabis Receptor Agonist–Induced Delirium^{b,c} (674)

Note: When pharmaceutical cannabis receptor agonist medication taken as prescribed. The designation “taken as prescribed” is used to differentiate medication-induced delirium from substance intoxication delirium.

F12.99 Unspecified Cannabis-Related Disorder (586)

Hallucinogen-Related Disorders (587)

____.____ Phencyclidine Use Disorder (587)

Specify if: In a controlled environment

Specify current severity/remission:

F16.10 Mild

F16.11 In early remission

F16.11 In sustained remission

F16.20 Moderate

F16.21 In early remission

F16.21 In sustained remission

F16.20 Severe

F16.21 In early remission

F16.21 In sustained remission

____.____ Other Hallucinogen Use Disorder (590)

Specify the particular hallucinogen

Specify if: In a controlled environment

Specify current severity/remission:

F16.10	Mild
F16.11	In early remission
F16.11	In sustained remission
F16.20	Moderate
F16.21	In early remission
F16.21	In sustained remission
F16.20	Severe
F16.21	In early remission
F16.21	In sustained remission
____.	Phencyclidine Intoxication (594)
F16.120	With mild use disorder
F16.220	With moderate or severe use disorder
F16.920	Without use disorder
____.	Other Hallucinogen Intoxication (596)
F16.120	With mild use disorder
F16.220	With moderate or severe use disorder
F16.920	Without use disorder

xliv

F16.983	Hallucinogen Persisting Perception Disorder (598)
____.	Phencyclidine-Induced Mental Disorders (600)
	Note: Disorders are listed in their order of appearance in the manual.
	^a <i>Specify</i> With onset during intoxication, With onset after medication use. Note: When prescribed as medication, substances in this class can also induce the relevant substance-induced mental disorder.
____.	Phencyclidine-Induced Psychotic Disorder ^a (126)
F16.159	With mild use disorder
F16.259	With moderate or severe use disorder
F16.959	Without use disorder
____.	Phencyclidine-Induced Bipolar and Related Disorder ^a (162)
F16.14	With mild use disorder
F16.24	With moderate or severe use disorder
F16.94	Without use disorder
____.	Phencyclidine-Induced Depressive Disorder ^a (201)
F16.14	With mild use disorder
F16.24	With moderate or severe use disorder

F16.94	Without use disorder
_____	Phencyclidine-Induced Anxiety Disorder ^a (255)
F16.180	With mild use disorder
F16.280	With moderate or severe use disorder
F16.980	Without use disorder
_____	Phencyclidine Intoxication Delirium (672)
	<i>Specify</i> if: Acute, Persistent
	<i>Specify</i> if: Hyperactive, Hypoactive, Mixed level of activity
F16.121	With mild use disorder
F16.221	With moderate or severe use disorder
F16.921	Without use disorder
_____	Hallucinogen-Induced Mental Disorders (600)
	Note: Disorders are listed in their order of appearance in the manual.
	^a <i>Specify</i> With onset during intoxication, With onset after medication use. Note: When prescribed as medication, substances in this class can also induce the relevant substance-induced mental disorder.
	^b <i>Specify</i> if: Acute, Persistent
	^c <i>Specify</i> if: Hyperactive, Hypoactive, Mixed level of activity
_____	Other Hallucinogen-Induced Psychotic Disorder ^a (126)
F16.159	With mild use disorder
F16.259	With moderate or severe use disorder
F16.959	Without use disorder
_____	Other Hallucinogen-Induced Bipolar and Related Disorder ^a (162)
F16.14	With mild use disorder
F16.24	With moderate or severe use disorder
F16.94	Without use disorder
_____	Other Hallucinogen-Induced Depressive Disorder ^a (201)
F16.14	With mild use disorder
F16.24	With moderate or severe use disorder
F16.94	Without use disorder

_____	Other Hallucinogen-Induced Anxiety Disorder ^a (255)
F16.180	With mild use disorder
F16.280	With moderate or severe use disorder

F16.980	Without use disorder
____.	Other Hallucinogen Intoxication Delirium ^{b,c} (672)
F16.121	With mild use disorder
F16.221	With moderate or severe use disorder
F16.921	Without use disorder
F16.921	Ketamine or Other Hallucinogen–Induced Delirium ^{b,c} (674)
	Note: When ketamine or other hallucinogen medication taken as prescribed. The designation “taken as prescribed” is used to differentiate medication-induced delirium from substance intoxication delirium.
F16.99	Unspecified Phencyclidine-Related Disorder (600)
F16.99	Unspecified Hallucinogen-Related Disorder (601)

Inhalant-Related Disorders (601)

____.	Inhalant Use Disorder (601)
	<i>Specify</i> the particular inhalant
	<i>Specify</i> if: In a controlled environment
	<i>Specify</i> current severity/remission:
F18.10	Mild
F18.11	In early remission
F18.11	In sustained remission
F18.20	Moderate
F18.21	In early remission
F18.21	In sustained remission
F18.20	Severe
F18.21	In early remission
F18.21	In sustained remission
____.	Inhalant Intoxication (605)
F18.120	With mild use disorder
F18.220	With moderate or severe use disorder
F18.920	Without use disorder
____.	Inhalant-Induced Mental Disorders (607)
	Note: Disorders are listed in their order of appearance in the manual.
	^a <i>Specify</i> With onset during intoxication
____.	Inhalant-Induced Psychotic Disorder ^a (126)
F18.159	With mild use disorder

F18.259	With moderate or severe use disorder
F18.959	Without use disorder
____.	Inhalant-Induced Depressive Disorder ^a (201)
F18.14	With mild use disorder
F18.24	With moderate or severe use disorder
F18.94	Without use disorder

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____.	Inhalant-Induced Anxiety Disorder ^a (255)
F18.180	With mild use disorder
F18.280	With moderate or severe use disorder
F18.980	Without use disorder
____.	Inhalant Intoxication Delirium (672)
	<i>Specify</i> if: Acute, Persistent
	<i>Specify</i> if: Hyperactive, Hypoactive, Mixed level of activity
F18.121	With mild use disorder
F18.221	With moderate or severe use disorder
F18.921	Without use disorder
____.	Inhalant-Induced Major Neurocognitive Disorder (712)
	<i>Specify</i> if: Persistent
F18.17	With mild use disorder
F18.27	With moderate or severe use disorder
F18.97	Without use disorder
____.	Inhalant-Induced Mild Neurocognitive Disorder (712)
	<i>Specify</i> if: Persistent
F18.188	With mild use disorder
F18.288	With moderate or severe use disorder
F18.988	Without use disorder
F18.99	Unspecified Inhalant-Related Disorder (608)
	Opioid-Related Disorders (608)
____.	Opioid Use Disorder (608)
	<i>Specify</i> if: On maintenance therapy, In a controlled environment
	<i>Specify</i> current severity/remission:
F11.10	Mild
F11.11	In early remission

F11.11	In sustained remission
F11.20	Moderate
F11.21	In early remission
F11.21	In sustained remission
F11.20	Severe
F11.21	In early remission
F11.21	In sustained remission
____.	Opioid Intoxication (615)
	Without perceptual disturbances
F11.120	With mild use disorder
F11.220	With moderate or severe use disorder
F11.920	Without use disorder
	With perceptual disturbances
F11.122	With mild use disorder
F11.222	With moderate or severe use disorder
F11.922	Without use disorder

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____.	Opioid Withdrawal (617)
F11.13	With mild use disorder
F11.23	With moderate or severe use disorder
F11.93	Without use disorder

____.	Opioid-Induced Mental Disorders (619)
	Note: Disorders are listed in their order of appearance in the manual.
	^a <i>Specify</i> With onset during intoxication, With onset during withdrawal, With onset after medication use. Note: When prescribed as medication, substances in this class can also induce the relevant substance-induced mental disorder.

^b*Specify* if: Acute, Persistent

^c*Specify* if: Hyperactive, Hypoactive, Mixed level of activity

____.	Opioid-Induced Depressive Disorder ^a (201)
F11.14	With mild use disorder
F11.24	With moderate or severe use disorder
F11.94	Without use disorder
____.	Opioid-Induced Anxiety Disorder ^a (255)
F11.180	With mild use disorder
F11.280	With moderate or severe use disorder

F11.980	Without use disorder
____.	Opioid-Induced Sleep Disorder ^a (468)
	<i>Specify</i> whether Insomnia type, Daytime sleepiness type, Mixed type
F11.182	With mild use disorder
F11.282	With moderate or severe use disorder
F11.982	Without use disorder
____.	Opioid-Induced Sexual Dysfunction ^a (504)
	<i>Specify</i> if: Mild, Moderate, Severe
F11.181	With mild use disorder
F11.281	With moderate or severe use disorder
F11.981	Without use disorder
____.	Opioid Intoxication Delirium ^{b,c} (672)
F11.121	With mild use disorder
F11.221	With moderate or severe use disorder
F11.921	Without use disorder
____.	Opioid Withdrawal Delirium ^{b,c} (673)
F11.188	With mild use disorder
F11.288	With moderate or severe use disorder
F11.988	Without use disorder
____.	Opioid-Induced Delirium ^{b,c} (674)
	Note: The designation “taken as prescribed” is used to differentiate medication-induced delirium from substance intoxication delirium and substance withdrawal delirium.
F11.921	When opioid medication taken as prescribed (674)
F11.988	During withdrawal from opioid medication taken as prescribed (674)
F11.99	Unspecified Opioid-Related Disorder (619)

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Sedative-, Hypnotic-, or Anxiolytic-Related Disorders (620)

____.	Sedative, Hypnotic, or Anxiolytic Use Disorder (620)
	<i>Specify</i> if: In a controlled environment
	<i>Specify</i> current severity/remission:
F13.10	Mild
F13.11	In early remission

F13.11	In sustained remission
F13.20	Moderate
F13.21	In early remission
F13.21	In sustained remission
F13.20	Severe
F13.21	In early remission
F13.21	In sustained remission
____.	Sedative, Hypnotic, or Anxiolytic Intoxication (626)
F13.120	With mild use disorder
F13.220	With moderate or severe use disorder
F13.920	Without use disorder
____.	Sedative, Hypnotic, or Anxiolytic Withdrawal (628)
	Without perceptual disturbances
F13.130	With mild use disorder
F13.230	With moderate or severe use disorder
F13.930	Without use disorder
	With perceptual disturbances
F13.132	With mild use disorder
F13.232	With moderate or severe use disorder
F13.932	Without use disorder
____.	Sedative-, Hypnotic-, or Anxiolytic-Induced Mental Disorders (631)
	Note: Disorders are listed in their order of appearance in the manual.
	^a <i>Specify</i> With onset during intoxication, With onset during withdrawal, With onset after medication use. Note: When prescribed as medication, substances in this class can also induce the relevant substance-induced mental disorder.
	^b <i>Specify</i> if: Acute, Persistent
	^c <i>Specify</i> if: Hyperactive, Hypoactive, Mixed level of activity
____.	Sedative-, Hypnotic-, or Anxiolytic-Induced Psychotic Disorder ^a (126)
F13.159	With mild use disorder
F13.259	With moderate or severe use disorder
F13.959	Without use disorder
____.	Sedative-, Hypnotic-, or Anxiolytic-Induced Bipolar and Related Disorder ^a (162)
F13.14	With mild use disorder

	With moderate or severe use disorder
F13.94	Without use disorder
____.____	Sedative-, Hypnotic-, or Anxiolytic-Induced Depressive Disorder ^a (201)
F13.14	With mild use disorder
F13.24	With moderate or severe use disorder
F13.94	Without use disorder
____.____	Sedative-, Hypnotic-, or Anxiolytic-Induced Anxiety Disorder ^a (255)
F13.180	With mild use disorder
F13.280	With moderate or severe use disorder
F13.980	Without use disorder
____.____	Sedative-, Hypnotic-, or Anxiolytic-Induced Sleep Disorder ^a (468)
	<i>Specify</i> whether Insomnia type, Daytime sleepiness type, Parasomnia type, Mixed type
F13.182	With mild use disorder
F13.282	With moderate or severe use disorder
F13.982	Without use disorder
____.____	Sedative-, Hypnotic-, or Anxiolytic-Induced Sexual Dysfunction ^a (504)
	<i>Specify</i> if: Mild, Moderate, Severe
F13.181	With mild use disorder
F13.281	With moderate or severe use disorder
F13.981	Without use disorder
____.____	Sedative-, Hypnotic-, or Anxiolytic Intoxication Delirium ^{b,c} (672)
F13.121	With mild use disorder
F13.221	With moderate or severe use disorder
F13.921	Without use disorder
____.____	Sedative-, Hypnotic-, or Anxiolytic Withdrawal Delirium ^{b,c} (673)
F13.131	With mild use disorder
F13.231	With moderate or severe use disorder
F13.931	Without use disorder
____.____	Sedative-, Hypnotic-, or Anxiolytic-Induced Delirium ^{b,c} (674)
	Note: The designation “taken as prescribed” is used to differentiate medication-induced delirium from substance intoxication delirium and substance withdrawal delirium.
F13.921	When sedative, hypnotic, or anxiolytic medication taken as prescribed

(674)

- F13.931** During withdrawal from sedative, hypnotic, or anxiolytic medication taken as prescribed (674)
- ____.____ Sedative-, Hypnotic-, or Anxiolytic-Induced Major Neurocognitive Disorder (712)
- Specify if:* Persistent
- F13.27** With moderate or severe use disorder
- F13.97** Without use disorder
- ____.____ Sedative-, Hypnotic-, or Anxiolytic-Induced Mild Neurocognitive Disorder (712)
- Specify if:* Persistent
- F13.188** With mild use disorder
- F13.288** With moderate or severe use disorder
- F13.988** Without use disorder
- F13.99** Unspecified Sedative-, Hypnotic-, or Anxiolytic-Related Disorder (632)

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Stimulant-Related Disorders (632)

- ____.____ Stimulant Use Disorder (632)
- Specify if:* In a controlled environment
- Specify current severity/remission:*
- ____.____ Mild
- F15.10** Amphetamine-type substance
- F14.10** Cocaine
- F15.10** Other or unspecified stimulant
- ____.____ Mild, In early remission
- F15.11** Amphetamine-type substance
- F14.11** Cocaine
- F15.11** Other or unspecified stimulant
- ____.____ Mild, In sustained remission
- F15.11** Amphetamine-type substance
- F14.11** Cocaine
- F15.11** Other or unspecified stimulant
- ____.____ Moderate
- F15.20** Amphetamine-type substance

F14.20	Cocaine
F15.20	Other or unspecified stimulant
____.____	Moderate, In early remission
F15.21	Amphetamine-type substance
F14.21	Cocaine
F15.21	Other or unspecified stimulant
____.____	Moderate, In sustained remission
F15.21	Amphetamine-type substance
F14.21	Cocaine
F15.21	Other or unspecified stimulant
____.____	Severe
F15.20	Amphetamine-type substance
F14.20	Cocaine
F15.20	Other or unspecified stimulant
____.____	Severe, In early remission
F15.21	Amphetamine-type substance
F14.21	Cocaine
F15.21	Other or unspecified stimulant
____.____	Severe, In sustained remission
F15.21	Amphetamine-type substance
F14.21	Cocaine
F15.21	Other or unspecified stimulant
____.____	Stimulant Intoxication (640)

Specify the particular intoxicant
Without perceptual disturbances

li

____.____	Amphetamine-type substance or other stimulant intoxication
F15.120	With mild use disorder
F15.220	With moderate or severe use disorder
F15.920	Without use disorder
____.____	Cocaine intoxication
F14.120	With mild use disorder
F14.220	With moderate or severe use disorder

F14.920	Without use disorder
____.____	With perceptual disturbances
____.____	Amphetamine-type substance or other stimulant intoxication
F15.122	With mild use disorder
F15.222	With moderate or severe use disorder
F15.922	Without use disorder
____.____	Cocaine intoxication
F14.122	With mild use disorder
F14.222	With moderate or severe use disorder
F14.922	Without use disorder
____.____	Stimulant Withdrawal (643)
____.____	<i>Specify</i> the particular substance that causes the withdrawal syndrome
____.____	Amphetamine-type substance or other stimulant withdrawal
F15.13	With mild use disorder
F15.23	With moderate or severe use disorder
F15.93	Without use disorder
____.____	Cocaine withdrawal
F14.13	With mild use disorder
F14.23	With moderate or severe use disorder
F14.93	Without use disorder
____.____	Stimulant-Induced Mental Disorders (644)
	Note: Disorders are listed in their order of appearance in the manual.
	^a <i>Specify</i> With onset during intoxication, With onset during withdrawal, With onset after medication use. Note: When prescribed as medication, amphetamine-type substances and other stimulants can also induce the relevant substance-induced mental disorder.
	^b <i>Specify</i> if: Acute, Persistent
	^c <i>Specify</i> if: Hyperactive, Hypoactive, Mixed level of activity
____.____	Amphetamine-Type Substance (or Other Stimulant)–Induced Psychotic Disorder ^a (126)
F15.159	With mild use disorder
F15.259	With moderate or severe use disorder
F15.959	Without use disorder
____.____	Cocaine-Induced Psychotic Disorder ^a (126)
F14.159	With mild use disorder
F14.259	With moderate or severe use disorder

F14.959 Without use disorder

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____.____ Amphetamine-Type Substance (or Other Stimulant)–Induced Bipolar and Related Disorder^a (162)

F15.14 With mild use disorder

F15.24 With moderate or severe use disorder

F15.94 Without use disorder

____.____ Cocaine-Induced Bipolar and Related Disorder^a (162)

F14.14 With mild use disorder

F14.24 With moderate or severe use disorder

F14.94 Without use disorder

____.____ Amphetamine-Type Substance (or Other Stimulant)–Induced Depressive Disorder^a (201)

F15.14 With mild use disorder

F15.24 With moderate or severe use disorder

F15.94 Without use disorder

____.____ Cocaine-Induced Depressive Disorder^a (201)

F14.14 With mild use disorder

F14.24 With moderate or severe use disorder

F14.94 Without use disorder

____.____ Amphetamine-Type Substance (or Other Stimulant)–Induced Anxiety Disorder^a (255)

F15.180 With mild use disorder

F15.280 With moderate or severe use disorder

F15.980 Without use disorder

____.____ Cocaine-Induced Anxiety Disorder^a (255)

F14.180 With mild use disorder

F14.280 With moderate or severe use disorder

F14.980 Without use disorder

____.____ Amphetamine-Type Substance (or Other Stimulant)–Induced Obsessive-Compulsive and Related Disorder^a (287)

F15.188 With mild use disorder

F15.288 With moderate or severe use disorder

F15.988	Without use disorder
____.____	Cocaine-Induced Obsessive-Compulsive and Related Disorder ^a (287)
F14.188	With mild use disorder
F14.288	With moderate or severe use disorder
F14.988	Without use disorder
____.____	Amphetamine-Type Substance (or Other Stimulant)–Induced Sleep Disorder ^a (468)
	<i>Specify</i> whether Insomnia type, Daytime sleepiness type, Mixed type
F15.182	With mild use disorder
F15.282	With moderate or severe use disorder
F15.982	Without use disorder
____.____	Cocaine-Induced Sleep Disorder ^a (468)
	<i>Specify</i> whether Insomnia type, Daytime sleepiness type, Mixed type
F14.182	With mild use disorder
F14.282	With moderate or severe use disorder
liii	
F14.982	Without use disorder
____.____	Amphetamine-Type Substance (or Other Stimulant)–Induced Sexual Dysfunction ^a (504)
	<i>Specify</i> if: Mild, Moderate, Severe
F15.181	With mild use disorder
F15.281	With moderate or severe use disorder
F15.981	Without use disorder
____.____	Cocaine-Induced Sexual Dysfunction ^a (504)
	<i>Specify</i> if: Mild, Moderate, Severe
F14.181	With mild use disorder
F14.281	With moderate or severe use disorder
F14.981	Without use disorder
____.____	Amphetamine-Type Substance (or Other Stimulant) Intoxication Delirium ^{b,c} (672)
F15.121	With mild use disorder
F15.221	With moderate or severe use disorder
F15.921	Without use disorder

____.____	Cocaine Intoxication Delirium ^{b,c} (672)
F14.121	With mild use disorder
F14.221	With moderate or severe use disorder
F14.921	Without use disorder
F15.921	Amphetamine-Type (or Other Stimulant) Medication–Induced Delirium ^{b,c} (674)
	Note: When amphetamine-type or other stimulant medication taken as prescribed. The designation “taken as prescribed” is used to differentiate medication-induced delirium from substance intoxication delirium.
____.____	Amphetamine-Type Substance (or Other Stimulant)–Induced Mild Neurocognitive Disorder (712)
	<i>Specify if:</i> Persistent
F15.188	With mild use disorder
F15.288	With moderate or severe use disorder
F15.988	Without use disorder
____.____	Cocaine-Induced Mild Neurocognitive Disorder (712)
	<i>Specify if:</i> Persistent
F14.188	With mild use disorder
F14.288	With moderate or severe use disorder
F14.988	Without use disorder
____.____	Unspecified Stimulant-Related Disorder (644)
F15.99	Amphetamine-type substance or other stimulant
F14.99	Cocaine
	Tobacco-Related Disorders (645)
____.____	Tobacco Use Disorder (645)
	<i>Specify if:</i> On maintenance therapy, In a controlled environment
	<i>Specify current severity/remission:</i>
Z72.0	Mild
	liv
F17.200	Moderate
F17.201	In early remission
F17.201	In sustained remission
F17.200	Severe
	In early remission

F17.201

F17.201 In sustained remission

F17.203 Tobacco Withdrawal (649)

Note: The ICD-10-CM code indicates the comorbid presence of a moderate or severe tobacco use disorder, which must be present in order to apply the code for tobacco withdrawal.

____.____ Tobacco-Induced Mental Disorders (651)

F17.208 Tobacco-Induced Sleep Disorder, With moderate or severe use disorder (468)

Specify whether Insomnia type, Daytime sleepiness type, Mixed type

Specify With onset during withdrawal, With onset after medication use

F17.209 Unspecified Tobacco-Related Disorder (651)

Other (or Unknown) Substance-Related Disorders (652)

____.____ Other (or Unknown) Substance Use Disorder (652)

Specify if: In a controlled environment

Specify current severity/remission:

F19.10 Mild

F19.11 In early remission

F19.11 In sustained remission

F19.20 Moderate

F19.21 In early remission

F19.21 In sustained remission

F19.20 Severe

F19.21 In early remission

F19.21 In sustained remission

____.____ Other (or Unknown) Substance Intoxication (656)

Without perceptual disturbances

F19.120 With mild use disorder

F19.220 With moderate or severe use disorder

F19.920 Without use disorder

With perceptual disturbances

F19.122 With mild use disorder

F19.222 With moderate or severe use disorder

F19.922 Without use disorder

____.____ Other (or Unknown) Substance Withdrawal (658)

Without perceptual disturbances

F19.130 With mild use disorder
F19.230 With moderate or severe use disorder
F19.930 Without use disorder

lv

F19.132 With perceptual disturbances
 With mild use disorder
F19.232 With moderate or severe use disorder
F19.932 Without use disorder

_____.____ Other (or Unknown) Substance–Induced Mental Disorders (660)

Note: Disorders are listed in their order of appearance in the manual.

^a*Specify* With onset during intoxication, With onset during withdrawal, With onset after medication use. **Note:** When prescribed as medication or taken over the counter, substances in this class can also induce the relevant substance-induced mental disorder.

^b*Specify* if: Acute, Persistent

^c*Specify* if: Hyperactive, Hypoactive, Mixed level of activity

_____.____ Other (or Unknown) Substance–Induced Psychotic Disorder^a (126)

F19.159 With mild use disorder
F19.259 With moderate or severe use disorder
F19.959 Without use disorder

_____.____ Other (or Unknown) Substance–Induced Bipolar and Related Disorder^a (162)

F19.14 With mild use disorder
F19.24 With moderate or severe use disorder
F19.94 Without use disorder

_____.____ Other (or Unknown) Substance–Induced Depressive Disorder^a (201)

F19.14 With mild use disorder
F19.24 With moderate or severe use disorder
F19.94 Without use disorder

_____.____ Other (or Unknown) Substance–Induced Anxiety Disorder^a (255)

F19.180 With mild use disorder
F19.280 With moderate or severe use disorder
F19.980 Without use disorder

_____.____ Other (or Unknown) Substance–Induced Obsessive-Compulsive and Related Disorder^a (287)

F19.188 With mild use disorder

F19.288	With moderate or severe use disorder
F19.988	Without use disorder
____.____	Other (or Unknown) Substance–Induced Sleep Disorder ^a (468) <i>Specify</i> whether Insomnia type, Daytime sleepiness type, Parasomnia type, Mixed type
F19.182	With mild use disorder
F19.282	With moderate or severe use disorder
F19.982	Without use disorder
____.____	Other (or Unknown) Substance–Induced Sexual Dysfunction ^a (504) <i>Specify</i> if: Mild, Moderate, Severe
F19.181	With mild use disorder
F19.281	With moderate or severe use disorder
F19.981	Without use disorder

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____.____	Other (or Unknown) Substance Intoxication Delirium ^{b,c} (672)
F19.121	With mild use disorder
F19.221	With moderate or severe use disorder
F19.921	Without use disorder
____.____	Other (or Unknown) Substance Withdrawal Delirium ^{b,c} (673)
F19.131	With mild use disorder
F19.231	With moderate or severe use disorder
F19.931	Without use disorder
____.____	Other (or Unknown) Medication–Induced Delirium ^{b,c} (674) Note: The designation “taken as prescribed” is used to differentiate medication-induced delirium from substance intoxication delirium and substance withdrawal delirium.
F19.921	When other (or unknown) medication taken as prescribed (674)
F19.931	During withdrawal from other (or unknown) medication taken as prescribed (674)
____.____	Other (or Unknown) Substance–Induced Major Neurocognitive Disorder (712) <i>Specify</i> if: Persistent
F19.17	With mild use disorder

F19.27	With moderate or severe use disorder
F19.97	Without use disorder
____.____	Other (or Unknown) Substance–Induced Mild Neurocognitive Disorder (712) <i>Specify</i> if: Persistent
F19.188	With mild use disorder
F19.288	With moderate or severe use disorder
F19.988	Without use disorder
F19.99	Unspecified Other (or Unknown) Substance–Related Disorder (660)
Non-Substance-Related Disorders (661)	
F63.0	Gambling Disorder (661) <i>Specify</i> if: Episodic, Persistent <i>Specify</i> if: In early remission, In sustained remission <i>Specify</i> current severity: Mild, Moderate, Severe

Neurocognitive Disorders (667)

____.____	Delirium (672) <i>Specify</i> if: Acute, Persistent <i>Specify</i> if: Hyperactive, Hypoactive, Mixed level of activity ^a Note: For applicable ICD-10-CM codes, refer to the substance classes under Substance-Related and Addictive Disorders for the specific substance/medication-induced delirium. See also the criteria set and corresponding recording procedures in the manual for more information. <i>Specify</i> whether:
____.____	Substance intoxication delirium ^a
____.____	Substance withdrawal delirium ^a

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____.____	Medication-induced delirium ^a
F05	Delirium due to another medical condition
F05	Delirium due to multiple etiologies
R41.0	Other Specified Delirium (678)
R41.0	Unspecified Delirium (678)

Major and Mild Neurocognitive Disorders (679)

Specify whether due to [any of the following medical etiologies]: Alzheimer’s disease, Frontotemporal degeneration, Lewy body disease, Vascular disease, Traumatic brain injury, Substance/medication use, HIV infection, Prion disease, Parkinson’s disease, Huntington’s disease, Another medical condition, Multiple etiologies, Unspecified etiology

^a*Specify* current severity: Mild, Moderate, Severe. *This specifier applies only to major neurocognitive disorders (including probable and possible).*

^b*Specify:* Without behavioral disturbance, With behavioral disturbance. *For all mild neurocognitive disorders, substance/medication-induced major neurocognitive disorder, and unspecified neurocognitive disorder, behavioral disturbance cannot be coded but should still be recorded.*

Note: As indicated for each subtype, an additional medical code is needed for most major neurocognitive disorders, including those due to probable and possible medical etiologies. The medical etiology should be coded first, before the code for the major neurocognitive disorder. An additional medical code should *not* be used for any mild neurocognitive disorder and is not used for major or mild vascular neurocognitive disorder, substance/medication-induced major or mild neurocognitive disorder, and unspecified neurocognitive disorder.

Coding note: *For major and mild neurocognitive disorders:* Use additional code(s) to indicate clinically significant psychiatric symptoms due to the same medical condition causing the major NCD (e.g., F06.2 psychotic disorder due to Alzheimer's disease with delusions; F06.32 depressive disorder due to Parkinson's disease, with major depressive-like episode.) **Note:** The additional codes for mental disorders due to another medical condition are included with disorders with which they share phenomenology (e.g., for depressive disorders due to another medical condition, see "Depressive Disorders").

Major or Mild Neurocognitive Disorder Due to Alzheimer's Disease (690)

____.____ Major Neurocognitive Disorder Due to Probable Alzheimer's Disease^a
Note: Code first **G30.9** Alzheimer's disease.

F02.81 With behavioral disturbance

F02.80 Without behavioral disturbance

____.____ Major Neurocognitive Disorder Due to Possible Alzheimer's Disease^a
Note: Code first **G30.9** Alzheimer's disease.

F02.81 With behavioral disturbance

F02.80 Without behavioral disturbance

G31.84 Mild Neurocognitive Disorder Due to Alzheimer's Disease^b

Major or Mild Frontotemporal Neurocognitive Disorder (695)

____.____ Major Neurocognitive Disorder Due to Probable Frontotemporal Degeneration^a
Note: Code first **G31.09** frontotemporal degeneration.

F02.81 With behavioral disturbance

F02.80 Without behavioral disturbance

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____.____ Major Neurocognitive Disorder Due to Possible Frontotemporal Degeneration^a
Note: Code first **G31.09** frontotemporal degeneration.

F02.81 With behavioral disturbance

F02.80 Without behavioral disturbance

G31.84 Mild Neurocognitive Disorder Due to Frontotemporal Degeneration^b

Major or Mild Neurocognitive Disorder With Lewy Bodies (699)

____.____ Major Neurocognitive Disorder With Probable Lewy Bodies^a
Note: Code first **G31.83** Lewy body disease.

F02.81 With behavioral disturbance

F02.80 Without behavioral disturbance

____.____ Major Neurocognitive Disorder With Possible Lewy Bodies^a
Note: Code first **G31.83** Lewy body disease.

F02.81 With behavioral disturbance

F02.80 Without behavioral disturbance

G31.84 Mild Neurocognitive Disorder With Lewy Bodies^b

Major or Mild Vascular Neurocognitive Disorder (702)

____.____ Major Neurocognitive Disorder Probably Due to Vascular Disease^a
Note: No additional medical code for vascular disease.

F01.51 With behavioral disturbance

F01.50 Without behavioral disturbance

____.____ Major Neurocognitive Disorder Possibly Due to Vascular Disease^a
Note: No additional medical code for vascular disease.

F01.51 With behavioral disturbance

F01.50 Without behavioral disturbance

G31.84 Mild Neurocognitive Disorder Due to Vascular Disease^b

Major or Mild Neurocognitive Disorder Due to Traumatic Brain Injury (706)

____.____ Major Neurocognitive Disorder Due to Traumatic Brain Injury^a
Note: For ICD-10-CM, code first **S06.2X9S** diffuse traumatic brain injury with loss of consciousness of unspecified duration, sequela.

F02.81 With behavioral disturbance

F02.80 Without behavioral disturbance

G31.84 Mild Neurocognitive Disorder Due to Traumatic Brain Injury^b

Substance/Medication-Induced Major or Mild Neurocognitive Disorder (712)

Note: No additional medical code. For applicable ICD-10-CM codes, refer to the substance classes under Substance-Related and Addictive Disorders for the specific substance/medication-induced major or mild neurocognitive disorder. See also the criteria set and corresponding recording procedures in the manual for more information.

Coding note: The ICD-10-CM code depends on whether or not there is a comorbid substance use disorder present for the same class of substance. In any case, an additional separate diagnosis of a substance use disorder is not given.

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Specify if: Persistent

____.____ Substance/Medication-Induced Major Neurocognitive Disorder^{a,b}
Note: If a substance use disorder is present, record mild substance use disorder (ICD-10-CM code not available if mild substance use disorder does not cause a major neurocognitive disorder) or moderate or severe substance use disorder; if no substance use disorder is present, record only [*specific substance*]-induced major neurocognitive disorder.

____.____ Substance/Medication-Induced Mild Neurocognitive Disorder^b
Note: If a substance use disorder is present, record mild substance use disorder or moderate or severe substance use disorder; if no substance use disorder is present, record only [*specific substance*]-induced mild neurocognitive disorder.

Major or Mild Neurocognitive Disorder Due to HIV Infection (717)

____.____ Major Neurocognitive Disorder Due to HIV Infection^a
Note: Code first **B20** HIV infection.

F02.81 With behavioral disturbance
F02.80 Without behavioral disturbance
G31.84 Mild Neurocognitive Disorder Due to HIV Infection^b
Major or Mild Neurocognitive Disorder Due to Prion Disease (721)

____.____ Major Neurocognitive Disorder Due to Prion Disease^a
Note: Code first **A81.9** prion disease.

F02.81 With behavioral disturbance
F02.80 Without behavioral disturbance

G31.84 Mild Neurocognitive Disorder Due to Prion Disease^b
Major or Mild Neurocognitive Disorder Due to Parkinson's Disease (723)

____.____ Major Neurocognitive Disorder Probably Due to Parkinson's Disease^a
Note: Code first **G20** Parkinson's disease.

F02.81 With behavioral disturbance
F02.80 Without behavioral disturbance

____.____ Major Neurocognitive Disorder Possibly Due to Parkinson's Disease^a
Note: Code first **G20** Parkinson's disease.

F02.81 With behavioral disturbance
F02.80 Without behavioral disturbance

G31.84 Mild Neurocognitive Disorder Due to Parkinson's Disease^b
Major or Mild Neurocognitive Disorder Due to Huntington's Disease (726)

____.____ Major Neurocognitive Disorder Due to Huntington's Disease^a
Note: Code first **G10** Huntington's disease.

F02.81 With behavioral disturbance
F02.80 Without behavioral disturbance

G31.84 Mild Neurocognitive Disorder Due to Huntington's Disease^b

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Major or Mild Neurocognitive Disorder Due to Another Medical Condition (729)

____.____ Major Neurocognitive Disorder Due to Another Medical Condition^a
Note: Code first the other medical condition.

F02.81 With behavioral disturbance
F02.80 Without behavioral disturbance

G31.84 Mild Neurocognitive Disorder Due to Another Medical Condition^b

Major or Mild Neurocognitive Disorder Due to Multiple Etiologies (731)

____.____ Major Neurocognitive Disorder Due to Multiple Etiologies^a
Note: Code first all the etiological medical conditions (with the exception of vascular disease, which is not coded).

F02.81	With behavioral disturbance
F02.80	Without behavioral disturbance
	Note: If vascular disease is among the multiple etiological medical conditions, code either F01.51 for major vascular neurocognitive disorder, with behavioral disturbance, or F01.50 for major vascular neurocognitive disorder, without behavioral disturbance.
G31.84	Mild Neurocognitive Disorder Due to Multiple Etiologies ^b
	Unspecified Neurocognitive Disorder ^b (732)
	Note: No additional medical code.
R41.9	Unspecified Neurocognitive Disorder ^b

Personality Disorders (733)

Cluster A Personality Disorders

F60.0	Paranoid Personality Disorder (737)
F60.1	Schizoid Personality Disorder (741)
F21	Schizotypal Personality Disorder (744)

Cluster B Personality Disorders

F60.2	Antisocial Personality Disorder (748)
F60.3	Borderline Personality Disorder (752)
F60.4	Histrionic Personality Disorder (757)
F60.81	Narcissistic Personality Disorder (760)

Cluster C Personality Disorders

F60.6	Avoidant Personality Disorder (764)
F60.7	Dependent Personality Disorder (768)
F60.5	Obsessive-Compulsive Personality Disorder (771)

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Other Personality Disorders

F07.0	Personality Change Due to Another Medical Condition (775) <i>Specify</i> whether: Labile type, Disinhibited type, Aggressive type, Apathetic type, Paranoid type, Other type, Combined type, Unspecified type
F60.89	Other Specified Personality Disorder (778)
F60.9	Unspecified Personality Disorder (778)

Paraphilic Disorders (779)

The following specifier applies to Paraphilic Disorders where indicated:

^a*Specify* if: In a controlled environment, In full remission

F65.3	Voyeuristic Disorder ^a (780)
F65.2	Exhibitionistic Disorder ^a (783) <i>Specify</i> whether: Sexually aroused by exposing genitals to prepubertal children, Sexually aroused by exposing genitals to physically mature individuals, Sexually aroused by exposing genitals to prepubertal children and to physically mature individuals
F65.81	Frotteuristic Disorder ^a (785)
F65.51	Sexual Masochism Disorder ^a (788) <i>Specify</i> if: With asphyxiophilia
F65.52	Sexual Sadism Disorder ^a (790)
F65.4	Pedophilic Disorder (792) <i>Specify</i> whether: Exclusive type, Nonexclusive type <i>Specify</i> if: Sexually attracted to males, Sexually attracted to females, Sexually attracted to both <i>Specify</i> if: Limited to incest
F65.0	Fetishistic Disorder ^a (796) <i>Specify</i> : Body part(s), Nonliving object(s), Other
F65.1	Transvestic Disorder ^a (798) <i>Specify</i> if: With fetishism, With autogynephilia
F65.89	Other Specified Paraphilic Disorder (801)
F65.9	Unspecified Paraphilic Disorder (801)

Other Mental Disorders and Additional Codes (803)

F06.8	Other Specified Mental Disorder Due to Another Medical Condition (803)
F09	Unspecified Mental Disorder Due to Another Medical Condition (804)
F99	Other Specified Mental Disorder (804)
F99	Unspecified Mental Disorder (805)
Z03.89	No Diagnosis or Condition (805)

Medication-Induced Movement Disorders and Other Adverse Effects of Medication (807)

_____	Medication-Induced Parkinsonism (807)
G21.11	Antipsychotic Medication– and Other Dopamine Receptor Blocking Agent–Induced Parkinsonism (807)
G21.19	Other Medication-Induced Parkinsonism (807)

G21.0	Neuroleptic Malignant Syndrome (810)
G24.02	Medication-Induced Acute Dystonia (812)
G25.71	Medication-Induced Acute Akathisia (813)
G24.01	Tardive Dyskinesia (814)
G24.09	Tardive Dystonia (816)
G25.71	Tardive Akathisia (816)
G25.1	Medication-Induced Postural Tremor (817)
G25.79	Other Medication-Induced Movement Disorder (818)
____.____	Antidepressant Discontinuation Syndrome (818)
T43.205A	Initial encounter
T43.205D	Subsequent encounter
T43.205S	Sequelae
____.____	Other Adverse Effect of Medication (819)
T50.905A	Initial encounter
T50.905D	Subsequent encounter
T50.905S	Sequelae

Other Conditions That May Be a Focus of Clinical Attention (821)

Suicidal Behavior and Nonsuicidal Self-Injury (822)

Suicidal Behavior (822)

____.____	Current Suicidal Behavior (822)
T14.91A	Initial encounter
T14.91D	Subsequent encounter
Z91.51	History of Suicidal Behavior (822)

Nonsuicidal Self-Injury (822)

R45.88	Current Nonsuicidal Self-Injury (822)
Z91.52	History of Nonsuicidal Self-Injury (822)

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Abuse and Neglect (822)

Child Maltreatment and Neglect Problems (823)

Child Physical Abuse (823)

____.____	Child Physical Abuse, Confirmed (823)
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T74.12XA	Initial encounter
T74.12XD	Subsequent encounter
____.____	Child Physical Abuse, Suspected (823)
T76.12XA	Initial encounter
T76.12XD	Subsequent encounter
____.____	Other Circumstances Related to Child Physical Abuse (823)
Z69.010	Encounter for mental health services for victim of child physical abuse by parent
Z69.020	Encounter for mental health services for victim of nonparental child physical abuse
Z62.810	Personal history (past history) of physical abuse in childhood
Z69.011	Encounter for mental health services for perpetrator of parental child physical abuse
Z69.021	Encounter for mental health services for perpetrator of nonparental child physical abuse
Child Sexual Abuse (823)	
____.____	Child Sexual Abuse, Confirmed (824)
T74.22XA	Initial encounter
T74.22XD	Subsequent encounter
____.____	Child Sexual Abuse, Suspected (824)
T76.22XA	Initial encounter
T76.22XD	Subsequent encounter
____.____	Other Circumstances Related to Child Sexual Abuse (824)
Z69.010	Encounter for mental health services for victim of child sexual abuse by parent
Z69.020	Encounter for mental health services for victim of nonparental child sexual abuse
Z62.810	Personal history (past history) of sexual abuse in childhood
Z69.011	Encounter for mental health services for perpetrator of parental child sexual abuse
Z69.021	Encounter for mental health services for perpetrator of nonparental child sexual abuse

Child Neglect (824)

____.____	Child Neglect, Confirmed (824)
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T74.02XA	Initial encounter
T74.02XD	Subsequent encounter
____.____	Child Neglect, Suspected (824)
T76.02XA	Initial encounter
T76.02XD	Subsequent encounter
____.____	Other Circumstances Related to Child Neglect (824)
Z69.010	Encounter for mental health services for victim of child neglect by parent
Z69.020	Encounter for mental health services for victim of nonparental child neglect
Z62.812	Personal history (past history) of neglect in childhood
Z69.011	Encounter for mental health services for perpetrator of parental child neglect
Z69.021	Encounter for mental health services for perpetrator of nonparental child neglect
Child Psychological Abuse (825)	
____.____	Child Psychological Abuse, Confirmed (825)
T74.32XA	Initial encounter
T74.32XD	Subsequent encounter
____.____	Child Psychological Abuse, Suspected (825)
T76.32XA	Initial encounter
T76.32XD	Subsequent encounter
____.____	Other Circumstances Related to Child Psychological Abuse (825)
Z69.010	Encounter for mental health services for victim of child psychological abuse by parent
Z69.020	Encounter for mental health services for victim of nonparental child psychological abuse
Z62.811	Personal history (past history) of psychological abuse in childhood
Z69.011	Encounter for mental health services for perpetrator of parental child psychological abuse
Z69.021	Encounter for mental health services for perpetrator of nonparental child psychological abuse
Adult Maltreatment and Neglect Problems (825)	
Spouse or Partner Violence, Physical (825)	
____.____	Spouse or Partner Violence, Physical, Confirmed (825)
T74.11XA	Initial encounter

T74.11XD Subsequent encounter

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____.____ Spouse or Partner Violence, Physical, Suspected (826)

T76.11XA Initial encounter

T76.11XD Subsequent encounter

____.____ Other Circumstances Related to Spouse or Partner Violence, Physical (826)

Z69.11 Encounter for mental health services for victim of spouse or partner violence, physical

Z91.410 Personal history (past history) of spouse or partner violence, physical

Z69.12 Encounter for mental health services for perpetrator of spouse or partner violence, physical

Spouse or Partner Violence, Sexual (826)

____.____ Spouse or Partner Violence, Sexual, Confirmed (826)

T74.21XA Initial encounter

T74.21XD Subsequent encounter

____.____ Spouse or Partner Violence, Sexual, Suspected (826)

T76.21XA Initial encounter

T76.21XD Subsequent encounter

____.____ Other Circumstances Related to Spouse or Partner Violence, Sexual (826)

Z69.81 Encounter for mental health services for victim of spouse or partner violence, sexual

Z91.410 Personal history (past history) of spouse or partner violence, sexual

Z69.12 Encounter for mental health services for perpetrator of spouse or partner violence, sexual

Spouse or Partner Neglect (826)

____.____ Spouse or Partner Neglect, Confirmed (826)

T74.01XA Initial encounter

T74.01XD Subsequent encounter

____.____ Spouse or Partner Neglect, Suspected (827)

T76.01XA Initial encounter

T76.01XD Subsequent encounter

____.____ Other Circumstances Related to Spouse or Partner Neglect (827)

Z69.11 Encounter for mental health services for victim of spouse or partner neglect

Z91.412 Personal history (past history) of spouse or partner neglect
Z69.12 Encounter for mental health services for perpetrator of spouse or partner neglect

Spouse or Partner Abuse, Psychological (827)

____.____ Spouse or Partner Abuse, Psychological, Confirmed (827)

T74.31XA Initial encounter

T74.31XD Subsequent encounter

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____.____ Spouse or Partner Abuse, Psychological, Suspected (827)

T76.31XA Initial encounter

T76.31XD Subsequent encounter

____.____ Other Circumstances Related to Spouse or Partner Abuse, Psychological (827)

Z69.11 Encounter for mental health services for victim of spouse or partner psychological abuse

Z91.411 Personal history (past history) of spouse or partner psychological abuse

Z69.12 Encounter for mental health services for perpetrator of spouse or partner psychological abuse

Adult Abuse by Nonspouse or Nonpartner (827)

____.____ Adult Physical Abuse by Nonspouse or Nonpartner, Confirmed (828)

T74.11XA Initial encounter

T74.11XD Subsequent encounter

____.____ Adult Physical Abuse by Nonspouse or Nonpartner, Suspected (828)

T76.11XA Initial encounter

T76.11XD Subsequent encounter

____.____ Adult Sexual Abuse by Nonspouse or Nonpartner, Confirmed (828)

T74.21XA Initial encounter

T74.21XD Subsequent encounter

____.____ Adult Sexual Abuse by Nonspouse or Nonpartner, Suspected (828)

T76.21XA Initial encounter

T76.21XD Subsequent encounter

____.____ Adult Psychological Abuse by Nonspouse or Nonpartner, Confirmed (828)

T74.31XA Initial encounter

T74.31XD Subsequent encounter

____.____	Adult Psychological Abuse by Nonspouse or Nonpartner, Suspected (828)
T76.31XA	Initial encounter
T76.31XD	Subsequent encounter
____.____	Other Circumstances Related to Adult Abuse by Nonspouse or Nonpartner (828)
Z69.81	Encounter for mental health services for victim of nonspousal or nonpartner adult abuse
Z69.82	Encounter for mental health services for perpetrator of nonspousal or nonpartner adult abuse

Relational Problems (828)

____.____	Parent-Child Relational Problem (829)
Z62.820	Parent–Biological Child (829)
Z62.821	Parent–Adopted Child (829)

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Z62.822	Parent–Foster Child (829)
Z62.898	Other Caregiver–Child (829)
Z62.891	Sibling Relational Problem (829)
Z63.0	Relationship Distress With Spouse or Intimate Partner (829)

Problems Related to the Family Environment (829)

Z62.29	Upbringing Away From Parents (829)
Z62.898	Child Affected by Parental Relationship Distress (830)
Z63.5	Disruption of Family by Separation or Divorce (830)
Z63.8	High Expressed Emotion Level Within Family (830)

Educational Problems (830)

Z55.0	Illiteracy and Low-Level Literacy (830)
Z55.1	Schooling Unavailable and Unattainable (830)
Z55.2	Failed School Examinations (830)
Z55.3	Underachievement in School (830)
Z55.4	Educational Maladjustment and Discord With Teachers and Classmates (830)
Z55.8	Problems Related to Inadequate Teaching (830)
Z55.9	Other Problems Related to Education and Literacy (830)

Occupational Problems (830)

Z56.82	Problem Related to Current Military Deployment Status (830)
Z56.0	Unemployment (831)

Z56.1	Change of Job (831)
Z56.2	Threat of Job Loss (831)
Z56.3	Stressful Work Schedule (831)
Z56.4	Discord With Boss and Workmates (831)
Z56.5	Uncongenial Work Environment (831)
Z56.6	Other Physical and Mental Strain Related to Work (831)
Z56.81	Sexual Harassment on the Job (831)
Z56.9	Other Problem Related to Employment (831)

Housing Problems (831)

Z59.01	Sheltered Homelessness (831)
Z59.02	Unsheltered Homelessness (831)
Z59.1	Inadequate Housing (831)
Z59.2	Discord With Neighbor, Lodger, or Landlord (831)
Z59.3	Problem Related to Living in a Residential Institution (831)
Z59.9	Other Housing Problem (831)

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Economic Problems (831)

Z59.41	Food Insecurity (832)
Z58.6	Lack of Safe Drinking Water (832)
Z59.5	Extreme Poverty (832)
Z59.6	Low Income (832)
Z59.7	Insufficient Social or Health Insurance or Welfare Support (832)
Z59.9	Other Economic Problem (832)

Problems Related to the Social Environment (832)

Z60.2	Problem Related to Living Alone (832)
Z60.3	Acculturation Difficulty (832)
Z60.4	Social Exclusion or Rejection (832)
Z60.5	Target of (Perceived) Adverse Discrimination or Persecution (832)
Z60.9	Other Problem Related to Social Environment (832)

Problems Related to Interaction With the Legal System (832)

Z65.0	Conviction in Criminal Proceedings Without Imprisonment (833)
Z65.1	Imprisonment or Other Incarceration (833)
	Problems Related to Release From Prison (833)

Z65.2

Z65.3 Problems Related to Other Legal Circumstances (833)

Problems Related to Other Psychosocial, Personal, and Environmental Circumstances (833)

Z72.9 Problem Related to Lifestyle (833)

Z64.0 Problems Related to Unwanted Pregnancy (833)

Z64.1 Problems Related to Multiparity (833)

Z64.4 Discord With Social Service Provider, Including Probation Officer, Case Manager, or Social Services Worker (833)

Z65.4 Victim of Crime (833)

Z65.4 Victim of Terrorism or Torture (833)

Z65.5 Exposure to Disaster, War, or Other Hostilities (833)

Problems Related to Access to Medical and Other Health Care (833)

Z75.3 Unavailability or Inaccessibility of Health Care Facilities (833)

Z75.4 Unavailability or Inaccessibility of Other Helping Agencies (833)

Circumstances of Personal History (833)

Z91.49 Personal History of Psychological Trauma (833)

Z91.82 Personal History of Military Deployment (833)

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Other Health Service Encounters for Counseling and Medical Advice (833)

Z31.5 Genetic Counseling (833)

Z70.9 Sex Counseling (834)

Z71.3 Dietary Counseling (834)

Z71.9 Other Counseling or Consultation (834)

Additional Conditions or Problems That May Be a Focus of Clinical Attention (834)

Z91.83 Wandering Associated With a Mental Disorder (834)

Z63.4 Uncomplicated Bereavement (834)

Z60.0 Phase of Life Problem (834)

Z65.8 Religious or Spiritual Problem (834)

Z72.811 Adult Antisocial Behavior (835)

Z72.810 Child or Adolescent Antisocial Behavior (835)

Z91.19 Nonadherence to Medical Treatment (835)

Overweight or Obesity (835)

E66.9

Z76.5

Malingering ([835](#))

R41.81

Age-Related Cognitive Decline ([835](#))

R41.83

Borderline Intellectual Functioning ([836](#))

SECTION I

DSM-5 Basics

Introduction

Use of the Manual

Cautionary Statement for Forensic Use of DSM-5

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This section provides a basic orientation to the purpose, structure, content, and use of DSM-5. The Introduction begins with a description of the DSM-5 and DSM-5-TR revision processes, followed by an overview of the DSM-5 organizational structure (e.g., regrouping of disorders, harmonization with ICD-11) and key conceptual issues, such as the definition of a mental disorder, categorical and dimensional approaches to diagnosis, cultural and social structural issues, and sex and gender differences. Use of the Manual features information to facilitate the use of DSM-5, such as a brief overview of the diagnostic process, use of subtypes and specifiers, other specified and unspecified mental disorder categories, use of clinical judgment, coding and recording procedures, notes about terminology, descriptions of the types of information in the DSM-5-TR text, and online enhancements. The section concludes with a Cautionary Statement for Forensic Use of DSM-5.

Introduction

The creation of the fifth edition of *Diagnostic and Statistical Manual of Mental Disorders* (DSM-5) was a massive undertaking that involved hundreds of people working toward a common goal over a 12-year period. Much thought and deliberation were involved in evaluating the diagnostic criteria, considering the organization of every aspect of the manual, and creating new features believed to be most useful to clinicians, such as dimensional measures that can help to identify emerging symptoms or determine and monitor changes in severity. All of these efforts have been directed toward the goal of enhancing the usefulness of DSM-5 as a clinical guide in the diagnosis of mental disorders, as well as its value for research.

DSM-5 strives to fulfill the need of clinicians, patients, families, and researchers for a clear and concise description of each mental disorder, which has been operationalized using diagnostic criteria that are supplemented by dimensional measures of severity and is accompanied by a digest of information about the diagnosis, including risk factors and culture- and sex- and gender-related issues.

Clinical training and experience are needed to use DSM for determining a clinical diagnosis. The diagnostic criteria identify symptoms and signs comprising affects, behaviors, cognitive functions, and personality traits along with physical signs, symptom combinations (syndromes), and durations that require clinical expertise to differentiate from normal variation and transient responses to stress. The diagnostic process can be facilitated by a thorough examination of the range of symptoms that might be present, such as by conducting a review of mental systems using the DSM-5 Level 1 Cross-Cutting Symptom Measure as recommended by The American Psychiatric Association (APA) *Practice Guidelines for the Psychiatric Evaluation of Adults* (see “Cross-Cutting Symptom Measures”).

The use of DSM criteria has the clear virtue of creating a common language for communication between clinicians about the diagnosis of disorders. Officially recognized disorders are located in Section II of the manual. However, it should be noted that these diagnostic criteria and their placement within the classification are based on current research and may need to be modified as research advances.

Development of DSM-5-TR

Brief History of Prior DSM Editions

The first edition of the American Psychiatric Association’s *Diagnostic and Statistical Manual of Mental Disorders* appeared in 1952. This was the first official manual of mental disorders to

contain a glossary of descriptions of the diagnostic categories. The use of the term “reaction” throughout the classification reflected the influence of Adolf Meyer’s psychobiological view that mental disorders represented reactions of the personality to psychological, social, and biological factors. In the development of the second edition (DSM-II), a decision was made to base the classification on the mental disorders section of

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the eighth revision of the *International Classification of Diseases* (ICD-8), for which representatives of the APA had provided consultation. Both DSM-II and ICD-8 went into effect in 1968.

As had been the case for DSM-I and DSM-II, the development of DSM-III was coordinated with the development of the *International Classification of Diseases*, specifically ICD-9, which was published in 1975 and implemented in 1978. Work began on DSM-III in 1974, with publication in 1980. DSM-III, under the direction of Robert L. Spitzer, M.D., introduced a number of important methodological innovations, including explicit diagnostic criteria and a descriptive approach that attempted to be neutral with respect to theories of etiology of mental disorders. Experience with DSM-III revealed a number of inconsistencies in the system. Therefore, the APA appointed a Workgroup to Revise DSM-III, which developed the revisions and corrections that led to DSM-III-R in 1987.

DSM-IV was published in 1994. It was the culmination of a 6-year effort that involved more than 1,000 individuals and numerous professional organizations. Much of the effort involved conducting a comprehensive review of the literature to establish a firm empirical basis for making modifications. Developers of DSM-IV and the tenth revision of the ICD worked closely to coordinate their efforts, resulting in increased congruence between the two systems. ICD-10 was published in 1992.

A thorough history of all DSM editions is located on the APA website: <https://www.psychiatry.org/psychiatrists/practice/dsm/history-of-the-dsm>.

DSM-5 Revision Process

In 1999, the APA launched an evaluation of the strengths and weaknesses of DSM. This effort was coordinated with the World Health Organization’s (WHO) Division of Mental Health, the World Psychiatric Association, and the National Institute of Mental Health (NIMH) in the form of several conferences, the proceedings of which were published in the 2002 monograph *A Research Agenda for DSM-V*. Thereafter, from 2003 to 2008, a cooperative agreement between the APA and WHO, supported by the NIMH, the National Institute on Drug Abuse (NIDA), and the National Institute on Alcohol Abuse and Alcoholism (NIAAA), led to the convening of 13 international DSM-5 research planning conferences, involving 400 participants from 39 countries, to review the world literature in specific diagnostic areas to prepare for revisions in developing both DSM-5 and the *International Classification of Diseases*, 11th Revision (ICD-11). Reports from these conferences formed the basis for future DSM-5 Task Force reviews and set the stage for the new edition of DSM.

In 2006, the APA named David J. Kupfer, M.D., as Chair and Darrel A. Regier, M.D., M.P.H., as Vice-Chair of the DSM-5 Task Force. They were charged with recommending chairs

for the 13 diagnostic work groups and additional task force members with a multidisciplinary range of expertise who would oversee the development of DSM-5. An additional vetting process was initiated by the APA Board of Trustees to disclose sources of income and thus avoid conflicts of interest by task force and work group members. The full disclosure of all income and research grants from commercial sources, including the pharmaceutical industry, in the previous 3 years, the imposition of an income cap from all commercial sources, and the publication of disclosures on a website set a new standard for the field. Thereafter, the task force of 28 members was approved in 2007, and appointments of more than 130 work group members were approved in 2008. More than 400 additional work group advisors with no voting authority were also approved to participate in the process. A clear concept of the next evolutionary stage for the classification of mental disorders was central to the efforts of the task force and the work groups. This vision emerged as the task force and work groups recounted the history of DSM-IV's classification, its current strengths and limitations, and strategic directions for its revision.

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An intensive 6-year process involved conducting literature reviews and secondary analyses, publishing research reports in scientific journals, developing draft diagnostic criteria, posting preliminary drafts on the DSM-5 website for public comment, presenting preliminary findings at professional meetings, performing field trials, and revising criteria and text. Overall, many health professional and educational groups were involved in this development and testing of DSM-5, including physicians, psychologists, social workers, nurses, counselors, epidemiologists, statisticians, neuroscientists, and neuropsychologists. Also, individuals with mental disorders and families of individuals with mental disorders, lawyers, consumer organizations, and advocacy groups participated in revising DSM-5 by providing feedback on the mental disorders described in this volume.

Proposals for Revisions

Proposals for the revision of DSM-IV diagnostic criteria were developed by members of the work groups on the basis of rationale, scope of change, expected impact on clinical management and public health, strength of the supporting research evidence, overall clarity, and clinical utility. Proposals encompassed changes to diagnostic criteria; the addition of new disorders, subtypes, and specifiers; and the deletion of existing disorders.

In the proposals for revisions, strengths and weaknesses in the current criteria and nosology were first identified. Novel scientific findings over the previous two decades were considered, leading to the creation of a research plan to assess potential changes through literature reviews and secondary data analyses. Four principles guided the draft revisions: 1) DSM-5 is primarily intended to be a manual to be used by clinicians, and revisions must be feasible for routine clinical practice; 2) recommendations for revisions should be guided by research evidence; 3) where possible, continuity should be maintained with previous editions of DSM; and 4) no a priori constraints should be placed on the degree of change between DSM-IV and DSM-5.

Building on the initial literature reviews, work groups identified key issues within their diagnostic areas. Work groups also examined broader methodological concerns, such as the presence of contradictory findings within the literature; development of a refined definition of

mental disorder; and a consideration of cross-cutting issues relevant to all disorders. Inclusion of a proposal for revision in Section II was informed by consideration of its advantages and disadvantages for public health and clinical utility, the strength of the evidence, and the magnitude of the change. New diagnoses and disorder subtypes and specifiers were subject to additional stipulations, such as demonstration of reliability (i.e., the degree to which two clinicians could independently arrive at the same diagnosis for a given patient). Disorders with low clinical utility and weak validity were considered for deletion. Placement of proposed conditions in “Conditions for Further Study” in Section III was contingent on the amount of empirical evidence generated on the proposed diagnosis, diagnostic reliability or validity, presence of clear clinical need, and potential benefit in advancing research.

DSM-5 Field Trials

The use of field trials to empirically demonstrate reliability was a noteworthy improvement introduced in DSM-III. The design and implementation strategy of the DSM-5 Field Trials represented several changes from approaches used for DSM-III and DSM-IV, particularly in obtaining data on the precision of kappa reliability estimates (a statistical measure that assesses level of agreement between raters that corrects for chance agreement due to prevalence rates) in the context of real-world clinical settings with high levels of diagnostic comorbidity. For DSM-5, field trials were extended by using two distinctive designs: one in large, diverse medical-academic settings and the other in routine clinical practices. The former capitalized on the need for large sample sizes to test hypotheses on

reliability and clinical utility of a range of diagnoses in a variety of patient populations; the latter supplied valuable information about how proposed revisions performed in everyday clinical settings among a diverse sample of DSM users. It is anticipated that future clinical and basic research studies will focus on the validity of the revised categorical diagnostic criteria and the underlying dimensional features of these disorders (including those now being explored by the NIMH Research Domain Criteria initiative).

The medical-academic field trials were conducted at 11 North American medical-academic sites from December 2010 to October 2011 and assessed the reliability, feasibility, and clinical utility of select revisions, with priority given to those that represented the greatest degree of change from DSM-IV or those potentially having the greatest public health impact. The full clinical patient populations coming to each site were screened for DSM-IV diagnoses or qualifying symptoms likely to predict several specific DSM-5 disorders of interest. Stratified samples of four to seven specific disorders, plus a stratum containing a representative sample of all other diagnoses, were identified for each site. Patients consented to the study and were randomly assigned for a clinical interview by a clinician blind to the clinical diagnosis, followed by a second interview that occurred within 2 weeks with a clinician blind to the diagnoses made by the first interviewer. Patients first filled out a computer-assisted inventory of cross-cutting symptoms in more than a dozen psychological domains. These inventories were scored by a computer, and results were provided to clinicians before they conducted a typical clinical interview (with no structured protocol). Clinicians were required to score the presence of

qualifying criteria on a computer-assisted DSM-5 diagnostic checklist, determine diagnoses, score the severity of the diagnosis, and upload the data to a central repository. This study design allowed the calculation of the degree to which two independent clinicians could agree on a diagnosis (using the intraclass kappa statistic) and on clinician-administered ratings of cross-cutting and diagnosis-specific symptom severity (using intraclass correlation coefficients), as well as the level of agreement on measures of self-reported cross-cutting symptoms, personality traits, disability, and diagnostic severity administered to the same patient on two occasions up to 2 weeks apart (using intraclass correlation coefficients), along with information on the precision of these estimates of reliability. It was also possible to assess the prevalence rates of both DSM-IV and DSM-5 conditions in the respective clinical populations.

The routine clinical practice field trials involved recruitment of individual psychiatrists and other mental health clinicians and were conducted from October 2011 to March 2012. A volunteer sample was recruited that included generalist and specialty psychiatrists, psychologists, licensed clinical social workers, counselors, marriage and family therapists, and advanced practice psychiatric mental health nurses. The field trials provided exposure of the proposed DSM-5 diagnoses and dimensional measures to a wide range of clinicians to assess their feasibility and clinical utility.

Public and Professional Review

In 2010, the APA launched a website devoted to the DSM-5 revision to facilitate public and professional input into DSM-5. All draft diagnostic criteria and proposed changes in organization were posted on www.dsm5.org for a 2-month comment period. Feedback totaled more than 8,000 submissions, which were systematically reviewed by each of the 13 work groups, whose members, where appropriate, integrated questions and comments into discussions of draft revisions and plans for field trial testing. After revisions to the initial draft criteria and proposed chapter organization, a second posting occurred in 2011. Work groups considered feedback from both web postings and the results of the DSM-5 Field Trials when drafting proposed final criteria, which were posted on the website for a third and final time in 2012. These three iterations of external review produced more than 13,000 individually signed comments on the website that were received and reviewed by the work

groups, plus thousands of organized petition signers for and against some proposed revisions, all of which allowed the task force to actively address concerns of DSM users, as well as patients and advocacy groups, and ensure that clinical utility remained a high priority.

Expert Review and Final Approval

The members of the 13 work groups, representing expertise in their respective areas, collaborated with advisors and reviewers under the overall direction of the DSM-5 Task Force to draft the diagnostic criteria and accompanying text. This effort was supported by a team of APA Division of Research staff and developed through a network of text coordinators from each work group. The preparation of the text was coordinated by the text editor, working in close collaboration

with the work groups and under the direction of the task force chairs. The Scientific Review Committee (SRC) was established to provide a scientific peer review process that was external to that of the work groups. The SRC chair, vice-chair, and six committee members were charged with reviewing the degree to which the proposed changes from DSM-IV could be supported with scientific evidence. Each proposal for diagnostic revision required a memorandum of evidence for change prepared by the work group and accompanied by a summary of supportive data organized around validators for the proposed diagnostic criteria (i.e., antecedent validators such as familial aggregation, concurrent validators such as biological markers, and prospective validators such as response to treatment or course of illness). The submissions were reviewed by the SRC and scored according to the strength of the supportive scientific data. Other justifications for change, such as those arising from clinical experience or need or from a conceptual reframing of diagnostic categories, were generally seen as outside the purview of the SRC. The reviewers' scores, which varied substantially across the different proposals, and an accompanying brief commentary were then returned to the APA Board of Trustees and the work groups for consideration and response.

The Clinical and Public Health Committee (CPHC), composed of a chair, vice-chair, and six members, was appointed to consider additional clinical utility, public health, and logical clarification issues for criteria that had not yet accumulated the type or level of evidence deemed sufficient for change by the SRC. This review process was particularly important for DSM-IV disorders with known deficiencies for which proposed remedies had neither been previously considered in the DSM revision process nor been subjected to replicated research studies. These selected disorders were evaluated by four to five external reviewers, and the blinded results were reviewed by CPHC members, who in turn made recommendations to the APA Board of Trustees and the work groups.

Forensic reviews of diagnostic criteria and text were conducted by the members of the APA Council on Psychiatry and Law for disorders frequently appearing in forensic environments and ones with high potential for influencing civil and criminal judgments in courtroom settings. Work groups also added forensic experts as advisors in pertinent areas to complement expertise provided by the Council on Psychiatry and Law.

A final recommendation from the task force was then provided to the APA Assembly's Committee on DSM-5 to consider some of the clinical utility and feasibility features of the proposed revisions. The Assembly is a deliberative body of the APA representing the district branches and wider membership that is composed of psychiatrists from throughout the United States who provide geographic, practice size, and interest-based diversity. The Committee on DSM-5 was composed of a diverse group of Assembly leaders.

Following the preceding review steps, executive "summit committee" sessions were held to consolidate input from review, including Assembly committee chairs, task force chairs, a forensic advisor, and a statistical advisor, for a preliminary review of each disorder by the Assembly and APA Board of Trustees executive committees. This preceded a preliminary review by the full APA Board of Trustees. The Assembly voted, in November 2012, to recommend that the Board approve the publication of DSM-5, and the APA Board

of Trustees approved its publication in December 2012. The many experts, reviewers, and

advisors who contributed to this process are listed in the Appendix.

Revisions to DSM-5

DSM-5 Iterative Revision Process

Advances in digital publishing that allow timely dissemination of changes have paved the way for the American Psychiatric Association to adopt an iterative improvement model for DSM, in which revisions are pegged to specific scientific advances. The DSM Steering Committee (analogous to the DSM-5 Task Force) was appointed in Spring 2014, with Paul S. Appelbaum, M.D., as Chair and Ellen Leibenluft, M.D., and Kenneth Kendler, M.D., as Vice Chairs, to oversee the iterative revision process, along with the establishment of a web portal (www.dsm5.org) to field proposals on a continuous basis. Proposed changes can include the addition of new disorders and deletion or modification of diagnostic criteria sets in Sections II and III of DSM-5, as well as changes to the text. Submissions must be accompanied by supportive information in a structured format, including the reasons for the change, the magnitude of change, data documenting improvements in validity across a range of validators, evidence of reliability and clinical utility, and consideration of current or potential deleterious consequences associated with the proposed change.

Approaches to validating diagnostic criteria for categorical mental disorders have included the following types of evidence: antecedent validators (similar genetic markers, family traits, temperament, and environmental exposure), concurrent validators (similar neural substrates, biomarkers, emotional and cognitive processing, and symptom similarity), and predictive validators (similar clinical course and treatment response). New criteria for current disorders are adopted if they produce improvement in some of these classes of validators. Furthermore, new disorders are added to DSM if they are shown to be valid by a substantial subset of these validators as well as meet the criteria for a mental disorder and demonstrate clinical utility.

Proposals submitted to the DSM web portal undergo an initial review by the Steering Committee to determine whether the evidence for the proposal, on its face, appears substantially likely to meet the criteria for approval. If so, the proposal is referred to one of the five standing Review Committees (functionally analogous to the DSM Work Groups), which cover broad domains of psychiatric diagnosis. On receipt of a proposal from the Steering Committee, the assigned Review Committee considers the evidence in support of the proposed change, requests additional information if necessary, and returns the proposal to the Steering Committee with recommendations for disposition and, in some cases, suggested modifications. If the Steering Committee concurs that sufficient evidence appears to exist in support of the proposal, the proposed revision is posted on the DSM-5 website for public comment. The final stage involves making necessary adjustments based on the comments and then forwarding the final version to the APA Assembly and Board of Trustees for their approval. Once approved, the online version of the manual (see <https://psychiatryonline.org>) is updated to reflect the changes. All changes that have been approved since the publication of DSM-5 in 2013 are included in DSM-5-TR.

DSM-5 Text Revision Process

In Spring 2019, APA started work on DSM-5-TR, with Michael B. First, M.D., and Philip Wang, M.D., Dr.P.H., as Revision Subcommittee Co-Chairs and Wilson M. Compton, M.D., and Daniel